

24 May 2023

Lucy Letby - Cross-examination Day 4: Babies E, G & H

CONTENTS

MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

Housekeeping

Wednesday, 24 May 2023

(10.30 am)

(Delay in proceedings)

(10.35 am)

(In the presence of the jury)

MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

MR JUSTICE GOSS: Sorry about the slight delay. Mr Johnson.

MR JOHNSON: Thank you, my Lord.

You may remember that at the beginning of each session I tend to ask you the same question, which is whether or not there is anything that you have already said that you wish to review in the light of the time that you've had to think about it.

LETBY: No.

JOHNSON: There's one thing that I would like to ask you arising out of something we've already discussed. Last Thursday, 18 May, you will remember I asked you a series of questions about the doctors that you believe have conspired against you to make things look worse --

LETBY: Yes.

JOHNSON: -- and you gave us four names.

LETBY: Yes.

JOHNSON: Drs Brearey, Gibbs, [Dr B] and Jayaram.

LETBY: Yes.

JOHNSON: One thing I didn't hear you say, but which I saw from reading the transcript, was that there was some input from somebody else, but the name didn't appear on the transcript and I didn't hear it. Do you remember saying that?

LETBY: No, that must be an error on my part. It's those four.

JOHNSON: All right. Let me just remind you of the questions and answers and see if it jogs your memory because it may be important.

The question I asked was:

"Question: Are you suggesting that there's some form of agreement between any of the medical staff who have given evidence in this case, some sort of agreement to get you?

"Answer: In a consultant group, yes, I do believe that.

"Question: Who's in the conspiracy group then?

"Answer: Which individuals?

"Question: Yes.

"Answer: I believe Ravi Jayaram, Stephen Brearey, [Dr B] and John Gibbs.

"Question: So the Gang of Four; yes?"

"Answer: "Yes, possibly with input from --"

And then the name, if you were about to give one, hasn't been recorded. So having had your memory jogged, were you about to give us another name?

LETBY: No.

JOHNSON: We're going to move on now to the case of [Baby E]; all right?

LETBY: Yes.

JOHNSON: Is it your case that staffing levels contributed to the collapses or death of [Baby E]?

LETBY: No.

JOHNSON: Is it your case that medical incompetence contributed to his collapse or death?

LETBY: Possibly, yes.

JOHNSON: Whose medical incompetence?

LETBY: The medical team that were on that night.

JOHNSON: And who in particular are you suggesting was incompetent?

LETBY: I don't think it's being incompetent, I just think collectively the doctors could have acted sooner to respond to his bleeding issue on reflection, seeing how much blood he had lost and what transfusion he was given or not given.

JOHNSON: So reacted sooner?

LETBY: Yes or treated the bleed.

JOHNSON: Well, of course their reaction would be dependent on when you told them there was an issue, wouldn't it?

LETBY: Yes.

JOHNSON: And you understand, don't you, that we, the prosecution, are suggesting that you didn't tell them as soon as [Baby E] did start to bleed?

LETBY: Yes.

JOHNSON: Because you will understand that it's the prosecution case that [Mother of Babies E & F] is telling the truth and that [Baby E] was bleeding at 21.00 hours?

LETBY: Yes.

JOHNSON: But you didn't tell anybody about that until at least an hour later, did you?

LETBY: No, I don't agree with that.

JOHNSON: When are you suggesting that something that wasn't done should have been done?

LETBY: I think once [Baby E] was profusely bleeding after 10 o'clock, maybe a blood transfusion or something could have been given sooner. I don't know if that would have made a difference to him.

JOHNSON: Right. To whom do you attribute that mistake?

LETBY: The medical team collectively. That's a medical decision.

JOHNSON: So Dr Harkness?

LETBY: And the other doctors. It was [Dr C], I believe, who was the consultant.

JOHNSON: Yes. Was there another doctor on that night?

LETBY: Only from his evidence, Dr Wood.

JOHNSON: Yes, Dr Wood, the SHO.

LETBY: Yes.

JOHNSON: And we'll come back to him soon. So one or a combination of those three medical practitioners?

LETBY: It's a medical decision, yes, to prescribe and give a blood transfusion, yes.

JOHNSON: During the evidence relating to [Baby E], do you remember that your friend Belinda Williamson gave evidence?

LETBY: Yes.

JOHNSON: And she was asked about the plumbing and the drains in the neonatal unit when she gave evidence on 16 November.

Do you remember those questions?

LETBY: I can't remember specifically, no.

JOHNSON: What have the plumbing and the drains got to do with the death of [Baby E]?

LETBY: I don't think I have ever said that they have got something to do with the death of [Baby E]. I think it's an important factor to note that there were often plumbing issues within the unit.

JOHNSON: What has that got to do with the death or the collapses of any of these children?

LETBY: I'm not saying that it is, but I think it's a contributing factor if the unit is dirty and staff are unable to wash their hands properly or we used to have raw sewage coming out of the sinks, running onto the floor in the intensive care nursery, and I think potentially that's not a safe working environment.

I don't know what effect that might have on a baby.

JOHNSON: Raw sewage coming out of the sinks in nursery 1?

LETBY: Yes.

JOHNSON: When did you first say that?

LETBY: I talked about there being sewage coming from the -- to the police.

JOHNSON: Will we find a Datix form with raw sewage coming out of the sink in nursery 1 on it?

LETBY: Not from myself, no.

JOHNSON: Or are you aware of anybody putting in a Datix form --

LETBY: I don't know, but we often had the plumbers in attending to the sink in nursery 1.

JOHNSON: So you're saying that the waste water was attached to, what, the soil pipe?

LETBY: Yes, we used to get backflow from the theatre sinks and -- in the CLS/labour ward theatre sinks we used to get sewage coming back through the sink, yes.

JOHNSON: But you never filled in a Datix form?

LETBY: Not personally, no.

JOHNSON: No. You did fill in a Datix form relating to the death of [Baby E] though, didn't you?

LETBY: Yes.

JOHNSON: I'd just like to have a look at that, please. It's tile 218. We'll click on it, please, Mr Murphy.

Just so that we understand, under what circumstances do you, and I'm talking about from your personal perspective now, under what circumstances do you fill in one of these forms?

LETBY: A Datix is completed as standard procedure if there's any death on the unit and then it's there for any other issues that I myself might want to raise that needs looking into, so any concerns I might have or anything that's gone against policy, anything that I think needs escalating to be reviewed by senior staff.

JOHNSON: You filled in this form, did you?

LETBY: I don't remember from memory, but I'm assuming this is.

JOHNSON: All right.

LETBY: I can't...

JOHNSON: As we've said many times, it isn't a memory test. Does the person who fills the form in, in effect, digitally sign it?

LETBY: Yes.

JOHNSON: And I think we'll see that on -- let's go to page 2, if we could, please, about a third of the way down. That's it.

So we see incident reporter there, don't we?

LETBY: Yes, so I filled in the form.

JOHNSON: Okay. And is there some way of knowing when this form was filled in?

LETBY: I think at the very top of the form, I think there's a date and time if I'm correct. I'm not...

JOHNSON: Okay. If Mr Murphy would oblige, please.

LETBY: So there it was submitted on the 4th at 05.53.

JOHNSON: So that's you opening it; is that right?

LETBY: That's the time the form is submitted, so that's when it was filed and saved.

JOHNSON: So this is an electronic process; is that right?

LETBY: Yes.

JOHNSON: And do you have to put in your personal PIN number or something to access the blank form?

LETBY: Yes.

JOHNSON: Just as we scroll down it, would you tell us which bits you filled in? So the ID and the name and the reference, are they --

LETBY: No, that's automatically generated by the computer.

JOHNSON: Even the name?

LETBY: Sorry? Yes, because it's part of the Meditech system.

JOHNSON: So, what, you log into [Baby E]'s individual case, is that right?

LETBY: Yes.

JOHNSON: And this list of data that we see there self-populates?

LETBY: From memory, I think so, yes.

JOHNSON: All right. So we go to "location", which is the second section. Is that self-populated as well?

LETBY: It's something I would fill in. It's a drop-down box that you have to select from.

JOHNSON: The coding section, next.

LETBY: Yes, that's me -- well, not the bottom two but the first four lines.

JOHNSON: So "Clinical incident, neonatal unit". It says "pick list". Is that where you're hitting a drop-down menu?

LETBY: Yes.

JOHNSON: The sub-category we see. We see this has got nothing to do with the result of staffing levels.

LETBY: No.

JOHNSON: Which is your selection?

LETBY: Yes.

JOHNSON: Risk grading. Is that what you put in?

LETBY: No, I'm not sure about that.

JOHNSON: It says "no harm caused", doesn't it?

LETBY: Yes.

JOHNSON: Which certainly doesn't make sense in the context of what we see in the next section. So "Details".

"Incident date" selected by you?

LETBY: Yes.

JOHNSON: The time of the incident, selected by you?

LETBY: Yes.

JOHNSON: The fact that it's:

"An unexpected death following a GI bleed full resus unsuccessful."

And then you have inserted the time of death; is that right?

LETBY: Yes.

JOHNSON: "Actions taken" and "Report to NRLS", is that done by you?

LETBY: No, the last input I have there is the "action taken".

Those bottom two rows, they're not entered by me, I'm not sure what they stand for.

JOHNSON: I think RIDDOR is something to do with the Health and Safety at Work Act, but we don't need to worry about that in the context of this case.

If we scroll down, please. The next section, is that completed by you?

LETBY: Yes. The duty of candour, yes.

JOHNSON: Then the incident investigation:

"Please use this field to document all updates in relation to this investigation."

That has somebody else's name against it?

LETBY: Yes.

JOHNSON: Is that anything to do with you?

LETBY: No.

JOHNSON: So patient details, plainly you?

LETBY: Yes. I'm not sure if they are automatically generated or if I've put those in, but yes.

JOHNSON: The next section, please, "Incident reporter", you?

LETBY: Yes.

JOHNSON: Then we have, "Linked claims", "Linked records".

Nothing to do with you because that's opened, we see, on 2 August 2017.

LETBY: And that's got somebody else's name as the handler, that's not myself.

JOHNSON: All right. Can we go down to the next bit, please, and we then have something to do with other documents, but I would just like to scroll down, please. And keep going.

SBAR, is this anything to do with you?

LETBY: No. This is what's been completed at the review meetings by the staff that were listed above.

JOHNSON: Then continuing down, please, there's quite a long history of what happened to [Baby E] and when.

LETBY: Yes.

JOHNSON: Nothing to do with you; is that right?

LETBY: No.

JOHNSON: And then can we keep going, please? And again, please.

All this, nothing to do with you; is that right?

LETBY: No.

JOHNSON: And then the "SI panel meeting". Again, nothing to do with you? That's a meeting that happened in August 2015.

LETBY: That's right.

JOHNSON: "SI tracker," again nothing to do with you?

LETBY: No.

JOHNSON: Can we continue? Any of this on the final page, anything to do with you?

LETBY: No.

JOHNSON: If we can continue. Can you just identify anything else that is anything to do with you?

LETBY: No.

JOHNSON: These are all just blanks, aren't they?

LETBY: They are, yes.

JOHNSON: If we keep going.

LETBY: That's not me.

JOHNSON: No.

LETBY: That's not me, no.

JOHNSON: No, okay. So does it come to this, therefore, that so far as your input is concerned, it's simply in effect reporting the death, so what is at the bottom of the very first page?

LETBY: Yes, so it's standard practice if a baby dies that we select that on a pick list and submit a Datix, yes.

JOHNSON: I want to just deal with [Baby E]'s course of treatment and presentation before he came under your care on that night shift of 3 August.

LETBY: Yes.

JOHNSON: If we can start with tile 13, please. You cared for [Baby E] on the night shift of the 1st into 2 August, didn't you?

LETBY: That says [Baby F].

JOHNSON: Yes, I know, but I think you --

LETBY: I can't say from memory.

JOHNSON: Right, okay. Certainly you were designated nurse for [Baby F] on the 1st and indeed on the subsequent night anyway?

LETBY: Yes.

JOHNSON: Do you remember sending a text on 2 August to one of your friends, saying, "Too Q word"?

LETBY: No.

JOHNSON: "Too [quiet]"?

LETBY: Not from memory, no.

JOHNSON: If we go to the [Baby F] sequence, please, to tile 26. I must have the wrong reference here. Well, we'll come back to it in the context of [Baby F].

Is "Too Q word", is that a sort of thing that you would send in a text?

LETBY: Yes, possibly, yes.

JOHNSON: And what does it mean?

LETBY: Q is -- we use -- the Q word is quiet.

JOHNSON: And so by "Too Q word", what did you mean by that?

LETBY: That the unit is quiet at that point.

JOHNSON: It's tile 26 in the [Baby E], sorry. It's my note, I've written the wrong name. If we go to that in the [Baby E] sequence, please, it's tile 26:

"Yeah, it's fine. Bit too Q word really."

Did you not like it when it was quiet?

LETBY: No, that's not what I'm saying. It's just sometimes it gets very quiet and we don't have many babies and you find you can be sitting around a lot and obviously we're there to help babies, so...

JOHNSON: So you don't like it when there's nothing to do?

LETBY: Well, there's always something to do, I didn't say there's nothing to do, but sometimes the shifts -- yes, sometimes they can be long nights if you haven't got as many babies on the unit, yes.

JOHNSON: So does it come to this then, that you enjoyed it when it was busy?

LETBY: I enjoyed being busy, yes.

JOHNSON: So rather than being a negative, being busy, so far as you are concerned being busy was a positive?

LETBY: I enjoyed being busy when it was managed, yes.

JOHNSON: We've just seen that on this particular shift -- so this is Sunday the 2nd into Monday, 3 August -- you were [Baby F]'s designated nurse --

LETBY: Yes.

JOHNSON: -- not [Baby E]'s.

LETBY: No, I don't believe so, no.

JOHNSON: Can we go to tile 30, please, on [Baby E]'s sequence? Can we just click on the -- go down to the bottom of the sheet, please.

That's your handwriting, isn't it?

LETBY: Yes.

JOHNSON: Is there any reason that you were telling the doctors that they had made a mistake on the calculations when you were not the designated nurse?

LETBY: Yes, because two nurses would have had to have verified that that prescription was wrong. So myself and another nurse would have had to have checked that.

JOHNSON: The other nurse on this shift was Mel Taylor, wasn't it?

LETBY: I can't recall that from memory.

JOHNSON: Well, that's what the evidence --

LETBY: Okay.

JOHNSON: -- shows. Why wasn't it -- Mel is senior to you, isn't she?

LETBY: She is.

JOHNSON: Why was she not speaking to the doctors given she was senior and this was her baby?

LETBY: It doesn't go on seniority. Two of you are responsible for signing a medication, therefore it's up to either of you to escalate any problems.

JOHNSON: Had you fallen out with Mel by this stage --

LETBY: No.

JOHNSON: -- because she wouldn't speak to you about [Baby A]'s death in the aftermath of his death?

LETBY: No, I hadn't fallen out with anyone, no.

JOHNSON: Is this an example, what we can see here, of the fact, of what Eirian Powell told us, that you were always prepared to call out other people's mistakes at the time?

LETBY: Yes.

JOHNSON: And that you were not afraid to confront the medical staff if you thought they had got it wrong?

LETBY: Yes.

JOHNSON: Were you very confident in your abilities?

LETBY: In my clinical competencies?

JOHNSON: Yes.

LETBY: Yes.

JOHNSON: Did you think that you were a cut above some of the other nurses, including Mel?

LETBY: No.

JOHNSON: Go to tile 61, please -- sorry, it's my fault, if we could go back to the previous part of this tile, please.

This is Melanie Taylor's note; do you see that?

LETBY: Yes.

JOHNSON: You see where she records:

"Mum and dad visiting at start of shift. Mum has been 2x with expressed breast milk overnight."

LETBY: Yes.

JOHNSON: That's what [Mother of Babies E & F] was doing, wasn't it, throughout this period? She was providing her own milk for her boys.

LETBY: Yes.

JOHNSON: Do you remember her telling the jury when she gave evidence that at the time she felt it was the only thing that she could do for them?

LETBY: Yes.

JOHNSON: And do you remember her always being very punctual in the provision of milk?

LETBY: I can't recall that now. A lot of parents on the unit expressed, so I can't recall specifically what her expressing habits were.

JOHNSON: If we go to tile 104 next, please. This is [Nurse B]'s notes from towards the end of the day shift, which immediately precedes the night shift on which you were [Baby E]'s designated nurse.

LETBY: Yes.

JOHNSON: This shows very good progress, doesn't it?

LETBY: Yes.

JOHNSON: And of course, this was day 6 of life for [Baby E].

LETBY: Yes.

JOHNSON: And he was doing very well, wasn't he?

LETBY: He is requiring insulin, so I wouldn't say he was very well, but yes, he is making progress with his feeds, yes.

JOHNSON: There was no suggestion, was there, prior to you coming on duty at 7.30 or 8 pm on this day that he had any gastrointestinal problems?

LETBY: No, that's right.

JOHNSON: And this was day 6 of life so far as he was concerned?

LETBY: Yes.

JOHNSON: Let's go to the population distribution chart for the night shift, please, on which he died. It's tiles 114 to 115. So that gives us, doesn't it, the list of staff that were working that night?

LETBY: Yes.

JOHNSON: And if we move on to 115, we see the population distribution; is that right?

LETBY: Yes.

JOHNSON: Do we see there that [Babies E & F] were the only occupants of nursery 1?

LETBY: Yes.

JOHNSON: That you were the only nurse in nursery 1?

LETBY: Yes.

JOHNSON: And that all the other babies were elsewhere being looked after by other staff?

LETBY: Yes.

JOHNSON: So other than people coming and going to collect equipment from nursery 1, you in effect had the nursery to yourself?

LETBY: I was the only nurse allocated babies, yes.

JOHNSON: Yes. Are you suggesting, just so that we understand, that, from a staffing point of view and from your own perspective, there was anything wrong with this arrangement?

LETBY: No.

JOHNSON: When you were being asked questions by your counsel back on, I think it was, 5 May, relating to events of this particular night, do you recall saying that you remembered that when [Mother of Babies E & F] appeared she was bringing milk?

LETBY: No, I can't recall what I said.

JOHNSON: Well, what's the truth of it? Is that the position, that when [Mother of Babies E & F] -- whatever time it is, and we'll come to that, but is it the position that whenever it was that [Mother of Babies E & F] appeared she was bringing breast milk?

LETBY: I don't recall from my memory now.

JOHNSON: Well, you said it. That's what you told the jury on 5 May.

LETBY: Okay.

JOHNSON: Is what you said true?

LETBY: I can't recall from my memory right here, right now.

JOHNSON: Right. So between -- so on 5 May 2023, you could remember that, an event that happened on 3 August 2015, so about 8 years earlier, but you're saying that since 5 May this year

you have forgotten that when [Mother of Babies E & F] appeared she was bringing --

LETBY: No, I don't think I've ever remembered specifically, but I've accepted that if [Mother of Babies E & F] says she was bringing milk down, that is a normal occurrence.

JOHNSON: I want to understand what you're saying about this. You are saying that if [Mother of Babies E & F] is saying she brought breast milk down on 3 August, then you accept that's a normal occurrence?

LETBY: Yes, it's a normal occurrence for mums to bring down expressed breast milk. I cannot say here and now what time [Mother of Babies E & F] brought milk down if that's what she did.

JOHNSON: I'm not asking you about the time. I want to be absolutely clear about that. We'll come to the time in a moment. What I'm asking you is what you remember about when [Mother of Babies E & F] appeared.

LETBY: I don't have any clear memory.

JOHNSON: Right, well, I will remind you. It's page 128 of the transcript of 5 May. These are questions being asked to you, asked of you, by Mr Myers. All right?

LETBY: Okay.

JOHNSON: I'm picking up, if anybody wants to check what I'm reading, at the bottom of page 127, which was towards the end of the day on 5 May.

This is Mr Myers speaking:

"You described her coming down and you made reference to Dr Harkness. Could you repeat what you were saying about that, please?"

And what you say is this:

"Dr Harkness was there when [Mother of Babies E & F] came down and she was updated by Dr Harkness about the blood that we'd found and the medications that we were there -- we were then starting to treat that."

And then this is the particular question and answer that I'm interested in. Mr Myers:

"Question: Do you recall why she'd come down?

"Answer: I don't recall specifically, no.

"Question: Did she have anything with her when she came down to the neonatal unit?"

Answer -- do you know what you said?

LETBY: She brought expressed breast milk down.

JOHNSON: "I think she brought breast milk down."

LETBY: Yes.

JOHNSON: "Question: Expressed breast milk?"

"Answer: Yes."

So no one was suggesting to you, were they --

LETBY: No, but I did say, "I think she brought expressed breast milk". I at no point was 100% certain that that is why she was there.

JOHNSON: That's your answer, is it?

LETBY: Yes.

JOHNSON: Why didn't you think that when I was asking the question about 2 or 3 minutes ago?

LETBY: Because it's the same thing: I can't be certain on either of them. I said "I think" to Mr Myers and today I'm saying I can't be sure.

JOHNSON: You understand, don't you, the significance of the bringing of the milk as far as the timing is concerned?

LETBY: Yes, I do.

JOHNSON: And you explain what it is, please.

LETBY: Sorry?

JOHNSON: Well, you understand it, so you explain what the significance is, please.

LETBY: The suggestion is that [Mother of Babies E & F] brought down milk at 21.00 and [Baby E] was bleeding at that point.

JOHNSON: Yes.

LETBY: I believe it was later in the evening.

JOHNSON: Well, I know that but what's the significance of 21.00 hours?

LETBY: She made a telephone call to her husband.

JOHNSON: Ah yes, but let's just park the phone call for a second.

What's the significance for [Baby E] of 21.00 hours?

LETBY: That's when [Mother of Babies E & F] thought she saw blood.

JOHNSON: But what was [Baby E] due at 21.00 hours?

LETBY: A feed.

JOHNSON: You know that, don't you?

LETBY: Yes.

JOHNSON: Why didn't you say that when I asked the question?

LETBY: I don't know what you mean.

JOHNSON: Really?

LETBY: Yes.

JOHNSON: Well, the significance is because you knew, and you know, because you heard [Mother of Babies E & F] say it from that very spot where you now sit, that she fixes the time of 21.00 hours not only by reference to the phone call that she made to her husband, [Father of Babies E & F], but also by the fact that [Baby E] was due milk at 21.00 hours.

LETBY: Yes.

JOHNSON: And that's why she said that she came down just before 21.00 hours.

LETBY: Yes.

JOHNSON: You understand all that, don't you?

LETBY: Yes.

JOHNSON: And that's why I'm suggesting to you the fact that she came down with milk and you remember it is part of the evidence that fixes the time at 21.00 hours; do you understand?

LETBY: I don't agree.

JOHNSON: You don't agree?

LETBY: No.

JOHNSON: I just want to understand what you are saying and what you do agree with. All right? You are saying, as I understand it at least, and please disagree if you wish, that the "16ml mucky aspirate", as you have described it --

LETBY: Yes.

JOHNSON: -- was taken before the 9 o'clock feed, before the 21.00 hours feed?

LETBY: Yes, it was.

JOHNSON: So we are agreed on that?

LETBY: Yes.

JOHNSON: And it would therefore follow, wouldn't it, that [Baby E] was due to be fed --

LETBY: That's right, yes.

JOHNSON: -- at 21.00 hours? Where was the milk coming from?

LETBY: So we have a milk fridge stored in nursery 1, so whenever parents bring down expressed breast milk, they put it in the fridge and it's ready then for any feeds that are happening.

JOHNSON: We've heard from a number of your colleagues about the system that was in place.

LETBY: Yes.

JOHNSON: How long before the delivery of the milk into the baby is the aspirate taken from the baby?

LETBY: Immediately before.

JOHNSON: Right. Well, where had the milk come from that you were about to give [Baby E] when you took that aspirate?

LETBY: So parents don't bring fresh milk for every feed.

[Mother of Babies E & F] may well have had milk gathering in the fridge, so if she's expressing more than [Baby E]'s requirements at that time then mums often have multiple bottles in the fridge.

JOHNSON: Well, that's a possibility, but I'm asking you what you remember.

LETBY: Well, I don't remember.

JOHNSON: You say, as I understand it, and again I'm just checking that I have understood things correctly, that as a result of obtaining that 16ml mucky aspirate, you showed it to Belinda Simcock?

LETBY: Yes.

JOHNSON: And that she and you agreed that the feed would be omitted?

LETBY: Yes. And that we needed to speak to the SHO.

JOHNSON: Yes. And the SHO in this context was Dr Wood?

LETBY: Yes.

JOHNSON: Do you remember him saying that if he'd ever been asked that question about omitting a feed, he would have made a note in [Baby E]'s medical records?

LETBY: Yes.

JOHNSON: There is no note, is there?

LETBY: No. We've seen before sometimes, doctors don't make notes when they said they would have.

JOHNSON: Well, there was Dr Mayberry and we heard about the circumstances in which Dr Mayberry didn't make a note, didn't we?

LETBY: Yes.

JOHNSON: And that he'd specifically asked, I think it was Sophie Ellis, was it --

LETBY: Yes.

JOHNSON: -- to make a note? And this was for one of the [Surname of Babies O, P & R] boys, I think, at about 7.30 one morning, just as shifts were handing over; is that right?

LETBY: Yes.

JOHNSON: What's the difference between Sophie Ellis' note and your note?

LETBY: For [Babies O & P]?

JOHNSON: Yes, her note for [Babies O & P] and your note in this case. Well, I'll tell you: the difference is that Sophie Ellis names the doctor, doesn't she? You don't name the doctor.

LETBY: No.

JOHNSON: No. If this really was what Dr Wood said or the advice he gave, why didn't you note his name?

LETBY: We don't always write the doctor's name. Sometimes we do just refer to them as SHO and registrar. There's only one SHO at night.

JOHNSON: But it makes it a bit more difficult to check, doesn't it, if anybody ever wants to check, if you don't know the name?

LETBY: No, if staff really wanted to know looking at -- reading my notes who the SHO was, they could look at the rota and see who was on. There would only be one SHO on at night.

JOHNSON: What time do you say that conversation happened?

LETBY: Some time after 21.00 when I got the aspirate back.

JOHNSON: Well, it would have to be. Thank you for that. But can you be a little more particular?

LETBY: No, I can't, no.

JOHNSON: You do understand, don't you, that we're suggesting this conversation never happened?

LETBY: Yes.

JOHNSON: At 22.00 hours, you wrote in your nursing note, albeit it wasn't written on the chart, that there was a large vomit of fresh blood?

LETBY: Yes.

JOHNSON: Why wasn't that written on the chart?

LETBY: That's an error on my part.

JOHNSON: A pretty important error though, isn't it?

LETBY: Yes, but it is documented in my notes and I believe Registrar Harkness was aware and was there when it happened.

JOHNSON: So you're saying that Dr Harkness was actually there when this vomit happened?

LETBY: Yes.

JOHNSON: So have I understood the position then, that this was a problem that first showed itself before the 9 o'clock due feed?

LETBY: Yes.

JOHNSON: And that it took you, therefore, about an hour actually to call for a doctor to check on this child?

LETBY: No, I don't agree with that.

JOHNSON: Well, when did you first call for a doctor to check on this child?

LETBY: I don't recall speaking to a doctor but I know myself and Belinda, we did -- it was discussed with an SHO on the telephone, I think. I don't know by who.

JOHNSON: Well, who was the doctor who was first actually called to see [Baby E]?

LETBY: Dr Harkness.

JOHNSON: When was he called?

LETBY: Once he'd had a bleed at 22.00.

JOHNSON: So an hour after a problem first manifested itself, according to you?

LETBY: Yes.

JOHNSON: If you had seen blood around [Baby E]'s mouth or his lips, what would you have done?

LETBY: If I'd seen blood at any point I would have escalated that to somebody.

JOHNSON: To who?

LETBY: To either the nurse in charge or one of the doctors.

JOHNSON: Do you agree that blood was never escalated to anybody until 22.00 hours?

LETBY: Can you say that again, please?

JOHNSON: Yes. Do you agree that blood was never escalated to anybody until 22.00 hours?

LETBY: Yes, because there wasn't blood prior to 22.00 hours.

JOHNSON: Well, that's one of the issues, isn't it, whether there was or there wasn't?

LETBY: Mm-hm.

JOHNSON: Yes. Was [Mother of Babies E & F] telling the truth about you?

LETBY: That's -- in what sense?

JOHNSON: In the sense of what she says you said to her at 9 pm, 21.00 hours?

MR MYERS: I hesitate to rise again --

MR JUSTICE GOSS: Well, this is the point: you can put to the witness what it is said [Mother of Babies E & F] said and then say, "Is that right or wrong?"

MR JOHNSON: Yes, I will, I'll do it that way.

[Mother of Babies E & F] says that when she came down to see her boys with milk at 21.00 hours, she saw [Baby E] with blood around his lips. Do you remember the picture that she drew?

LETBY: Yes.

JOHNSON: Perhaps just to remind us all and to remind you, we'll look at the picture, please. It's J2434.

So that was [Mother of Babies E & F]'s best effort to replicate in visual form what it was she saw.

LETBY: Yes.

JOHNSON: This has got nothing to do with blood coming up an NG tube, has it?

LETBY: No.

JOHNSON: No, because if it came up an NG tube it couldn't end up on the lips, could it?

LETBY: No.

JOHNSON: No. So we can exclude as a possibility that this has got anything to do with blood coming up an NG tube; do you agree?

LETBY: Yes.

JOHNSON: Did you ever see anything like that?

LETBY: [Baby E] did have blood around his mouth, yes, after 22.00.

JOHNSON: After the vomit?

LETBY: I can't recall specific times, but he did have blood on his face, yes.

JOHNSON: Well, we've dealt with your note or a note of the vomit.

Is that when you are saying [Baby E] had blood like that?

LETBY: There was no blood prior to that, so yes.

JOHNSON: Is [Mother of Babies E & F] mistaken when she says that when she went down, other than her boys, you were the only person there?

LETBY: No, I accept that. I don't have any memory, but I was allocated to look after both of them in nursery 1, so yes.

JOHNSON: It's just that at other times you've suggested that when [Mother of Babies E & F] came down, Dr Harkness was there?

LETBY: Yes, he was there at 22.00.

JOHNSON: So I'll ask the question again then: is [Mother of Babies E & F] mistaken when she says that when she came down, you were the only person other than her boys that were there?

MR MYERS: Again, this is exactly the point. The question, if I may say, is, "What do you say happened?" To put what [Mother of Babies E & F] said and then ask the witness what happened. To keep on talking in terms of whether they were mistaken or

whether they were lying is to draw the defendant into commenting on their evidence in a way that shouldn't be done. It's her explanation that matters.

MR JUSTICE GOSS: Mr Johnson, you can ask the question in response to what the witness has just said as to when it was that she accepts she was on her own --

MR JOHNSON: Yes.

MR JUSTICE GOSS: -- and whether it was an occasion when [Mother of Babies E & F] came down that she was on her own.

MR JOHNSON: When do you say that you were there alone?

LETBY: When [Mother of Babies E & F] was present on the handover, so shortly -- around 8 pm.

JOHNSON: There was no question of any blood at that point, was there?

LETBY: No.

JOHNSON: You're not telling the truth about that, are you?

LETBY: Yes, I am.

JOHNSON: I'm going to suggest to you that when [Mother of Babies E & F] came down at 21.00 hours, you had inflicted an injury on [Baby E] to cause bleeding.

LETBY: No, I do not accept that. That did not happen.

JOHNSON: And that is why, as she describes it, he was screaming?

LETBY: No.

JOHNSON: Did you at any stage ever fall out with [Mother of Babies E & F]?

LETBY: No.

JOHNSON: Just looking at that picture, have you ever seen blood on any other baby at any time like that?

LETBY: In my career?

JOHNSON: Yes.

LETBY: No.

JOHNSON: So can we take it, therefore, that to see blood on a baby like this is wholly exceptional?

LETBY: Yes.

JOHNSON: Did you tell [Mother of Babies E & F] that the source of the blood was the insertion of the nasogastric tube?

LETBY: No.

JOHNSON: Have you ever told anyone that a nasogastric tube can cause a small amount of blood?

LETBY: Yes, it can cause a small amount of irritation when it's first being inserted, yes.

JOHNSON: And how much blood does it produce?

LETBY: Very minimal.

JOHNSON: About a millilitre of blood?

LETBY: I couldn't be specific, but it would be a small amount mixed in with stomach acid.

JOHNSON: This is blood in the mouth that I'm talking about, just so that you understand. Is that what you're agreeing with?

LETBY: No, I'm agreeing that a nasogastric tube irritation can cause some blood to come back from the NG tube.

JOHNSON: From the tube?

LETBY: Yes.

JOHNSON: Not into the mouth?

LETBY: No.

JOHNSON: Do you remember telling the police in your interview in the case of [Baby N] that NGTs can cause bleeding?

LETBY: Yes.

JOHNSON: So do you agree that it is the sort of thing that you do say, that you have said?

LETBY: Yes, I haven't said that it causes blood around the mouth but it does cause blood, yes, sometimes, with trauma, yes.

JOHNSON: And that is what you told [Mother of Babies E & F], wasn't it --

LETBY: No.

JOHNSON: -- when she queried why it was her son had blood round his mouth?

LETBY: No, I don't recall saying that.

JOHNSON: Well, you say you don't recall saying that. Are you saying --

LETBY: I don't believe I would have said that.

JOHNSON: Although it is something that you have said to the police?

LETBY: No, I haven't said it -- what do you mean, sorry?

JOHNSON: Well, you have said that NGTs cause bleeding.

LETBY: Bleeding orally?

JOHNSON: Yes.

LETBY: Okay.

JOHNSON: But you definitely didn't say it on this occasion?

LETBY: No.

JOHNSON: You are lying about that, aren't you?

LETBY: No.

JOHNSON: Is it your view that any baby could have a bleed like [Baby E]?

LETBY: Do you mean is it medically possible?

JOHNSON: I'm asking whether it is your view that any baby can have a bleed like [Baby E].

LETBY: Well, medically speaking, yes.

JOHNSON: That's what you said to the police, wasn't it? Any baby can have a bleed like [Baby E].

LETBY: Yes.

JOHNSON: Do you remember saying that in interview?

LETBY: Yes.

JOHNSON: Do you remember saying as much to your pal, Jennifer Jones-Key, in a text not very long after [Baby E] had died?

LETBY: Yes.

JOHNSON: Just go to tile 253, please.

They're your words, aren't they?

LETBY: Yes.

JOHNSON: Had you ever seen anything like this before [Baby E]?

LETBY: Not a gastric haemorrhage, no.

JOHNSON: Ever seen one after [Baby E]?

LETBY: Not gastric, no.

JOHNSON: What was your basis for saying that any baby could have suffered this?

LETBY: Medically speaking, at the time, we thought [Baby E] had NEC and a risk of that is bleeding.

JOHNSON: Have you seen cases of NEC?

LETBY: Yes.

JOHNSON: Have you ever seen this sort of bleeding?

LETBY: No, I haven't, no.

JOHNSON: So I ask again: what was your basis for saying that any baby could have had bleeding like this?

LETBY: Because any baby could have had the condition that [Baby E] had.

JOHNSON: I would like to go, in the light of what you have told us, to your defence statement, please, which I think is still on the table in front of you there. Can we go to the section that relates to [Baby E]? It's paragraph 67 onwards and it's at page 9.

There you say:

"I did nothing to harm [Baby E]."

LETBY: Yes.

JOHNSON: "I have not falsified any records."

LETBY: No.

JOHNSON: And you understand, don't you, that we, the prosecution, are suggesting that you have falsified the records in this particular case?

LETBY: Yes.

JOHNSON: And in particular, relating to the time at which [Baby E] started to bleed?

LETBY: Yes.

JOHNSON: What I'd like you to do, please, is to read out paragraph 69 so that the jury can hear what it was you were saying in February 2021.

LETBY: "When I saw [Mother of Babies E & F] on the evening of 3 August, she had come down with some expressed milk."

JOHNSON: Just pause there:

"She'd come down with some expressed milk."

LETBY: Yes.

JOHNSON: Why were you so reticent before when I was asking you about that?

LETBY: Because I don't remember that sitting here now.

JOHNSON: This is something you did remember in February 2021?

LETBY: Yes.

JOHNSON: Can you carry on, please?

LETBY: "I think this may have been later than 21.00."

JOHNSON: Pausing there, "This may have been later than 21.00".

LETBY: Yes.

JOHNSON: You're now saying it cannot have been before 22.00?

LETBY: No, that's -- I think [Mother of Babies E & F] came down at 22.00.

JOHNSON: Your case now, correct me if I've misunderstood it, because of the time you say [Baby E] started to bleed, is [Mother of Babies E & F] cannot have come down to nursery 1 before 22.00. Have I understood that correctly?

LETBY: No, I can't say that definitively. I know that there was no blood prior to 22.00.

JOHNSON: Yes. So why do you say -- when [Mother of Babies E & F] came down, why do you say, "I think this may have been later than 21.00 hours"?

LETBY: Because I believed at the time that it may have been later than 21.00.

JOHNSON: Why didn't you say, "[Mother of Babies E & F] didn't come down before 22.00 hours, she's wrong when she said she came down at 21.00 hours"?

LETBY: Because I don't think with 100% certainty that I can say that.

JOHNSON: But that's your case, isn't it, Lucy Letby?

LETBY: No, my case is that there was no blood prior to 22.00.

I don't remember [Mother of Babies E & F] coming down prior to that.

JOHNSON: "I do not recall any blood around [Baby E]'s mouth when she came down (inaudible)"; is that right?

LETBY: Yes:

"Nor was there any conversation about a tube irritating him."

JOHNSON: The next sentence, please?

LETBY: "I did say a doctor was coming to see him. I did not send [Mother of Babies E & F] away. If [Mother of Babies E & F] ever did see blood on [Baby E]'s mouth it must have been at a later time than 21.00 and after blood came down the tube on free drainage."

JOHNSON: Yes. We have already established and you have agreed that the blood on [Baby E]'s mouth can't be the blood that comes up the nasogastric tube.

LETBY: No.

JOHNSON: So how do we square that with what you've just read:

"If [Mother of Babies E & F] ever did see blood on [Baby E]'s mouth, it must have been at a later time than 21.00 hours after blood came down the tube on free drainage"?

LETBY: Yes, so it's after he vomited and his tube was on free drainage.

JOHNSON: That's not what you say in your defence statement though, is it?

LETBY: It is: I'm saying after blood came down the tube on free drainage.

JOHNSON: No, but you've just added the little bit "he vomited".

LETBY: Yes.

JOHNSON: It's not what you say in the defence statement, is it?

LETBY: No, I don't mention the vomit there, no.

JOHNSON: No. Did you understand when you wrote this document that if you failed to mention in it something which you later mentioned in court, a jury might conclude that you've made it up since you wrote this document?

LETBY: I -- I can't say I recall writing this document.

JOHNSON: And that is what you're doing, isn't it, you are lying about the detail of what you say happened with [Baby E]?

LETBY: No.

JOHNSON: Let's move on to the next paragraph, please, paragraph 70:

"I do recall a mucky aspirate in his NG tube. There was 16ml of bile that needed to be aspirated."

That's not what you are saying now, is it?

LETBY: Yes.

JOHNSON: No, no. Bile-stained is what you now say, isn't it?

LETBY: No, I've only ever referred to it as a mucky aspirate, which is bile.

JOHNSON: Why not 16ml of bile?

LETBY: It's just terminology that we use. We often refer to bile aspirates as mucky.

JOHNSON: Are you seriously suggesting that you aspirated 16ml of bile from a six-day old child?

LETBY: I haven't said it's frank bile, but yeah, a cumulative total, that was accurate, yes, 16ml.

JOHNSON: So you're saying it's not bile-stained, it's bile?

Is that what you say in the defence statement?

LETBY: I say I recall a mucky aspirate and there was 16ml of bile, so yes, there was bile within the mucky aspirate, yes.

JOHNSON: This is something that we can go to it if you want, but this is something you record in the nursing note as a bile-stained aspirate; do you remember that?

LETBY: Yes.

JOHNSON: There's a difference, isn't there, between bile-stained and bile?

LETBY: Yes.

JOHNSON: Why the difference?

LETBY: I can't recall. This says mucky aspirate and that's what I remember it to be.

JOHNSON: No, no.

LETBY: It's an error then if this says bile.

JOHNSON: This says "16ml of bile".

LETBY: Mm. Well, that's an error then.

JOHNSON: Yes. But why the error?

LETBY: Perhaps there'd been a misunderstanding of what mucky means.

JOHNSON: No, no, these are your words:

"There was 16ml of bile."

Why did you say that in this defence statement?

LETBY: I don't know.

JOHNSON: Can we go right to the end of it? I did read this to you last Wednesday, but -- the very final page, page 28.

Could you just read out that italicized paragraph, please?

LETBY: "This defence statement is not signed. The defendant made additional amendments to the statement at a meeting on 11 February 2022. During this conference, however, the defendant accepted this document and the amendments as being an accurate summary of the case. The court and the prosecution will be provided with a signed copy of this document in due course."

JOHNSON: Was that true? Is that statement true?

LETBY: Yes.

JOHNSON: So if this was your case as at February last year, why has your case changed?

LETBY: I don't think it's changed. I think there's just been some clarification with some of the points.

JOHNSON: You are lying, aren't you, Lucy Letby?

LETBY: No.

JOHNSON: Paragraph 75, please, of the defence statement. What does that say?

LETBY: "After [Baby E] had died, I found blood in his nappy."

JOHNSON: Have you said that in evidence yet?

LETBY: I don't recall.

JOHNSON: I'm going to suggest you haven't.

LETBY: Okay.

JOHNSON: Is it true?

LETBY: Yes.

JOHNSON: What did you do about it?

LETBY: It was after death. When we went to bath him after his death there was blood in the nappy.

JOHNSON: The question, though, is: what did you do about it?

LETBY: It's written in my nursing notes. I don't recall doing anything else about it because [Baby E] was deceased at the time.

JOHNSON: And which nursing note did you write it in?

LETBY: I think I've written it in my nursing notes.

JOHNSON: Since we last met, have you been looking at the evidence in [Baby E]'s case?

LETBY: Yes.

JOHNSON: Have you been looking at your nursing notes, anticipating that I might ask you a question or two about them?

LETBY: Yes.

JOHNSON: Does it say in the nursing notes that there was blood in the nappy?

LETBY: I can't recall.

JOHNSON: Oh, I think you can. Why don't you tell the jury?

LETBY: No, I can't remember.

JOHNSON: Let me give you a copy of your nursing notes. My Lord, I am going to give a copy of this to the jury as well, please, because what we're going to do is to compare what's in the nursing notes to what's in some other documents, and having multiple documents on the screen is going to be a bit difficult. Perhaps out of fairness to the witness, I can hand her a copy now, she can read it during the break, and when we come back we can deal with this point.

MR JUSTICE GOSS: Right. If the usher could just hand it to the officer.

(Handed)

All right. So you have an opportunity now to read through that so that you can refresh your memory from what's in that document. And we'll have it when we resume in 15 minutes, please, members of the jury.

(11.41 am)

(A short break)

(11.56 am)

MR JUSTICE GOSS: Mr Johnson, can these be distributed?

MR JOHNSON: Yes, please.

I'm going to invite you to put these behind divider 5 of jury bundle 2, but it might be easier just to keep them out for now. I always forget at the end to ask you to put them away, so that's where they go.

(Pause)

MR JUSTICE GOSS: Can you just remind us which tile this is in the sequence of events?

MR JOHNSON: I can.

MR JUSTICE GOSS: Or perhaps Mr Astbury or Mr Driver.

MR JOHNSON: I can do it. It is, as always, on multiple tiles, but it's on 116, 126, 127, 139, et cetera. So 116 is the beginning.

MR JUSTICE GOSS: Right. Thank you.

MR JOHNSON: It may make sense -- I can hear people are putting it away -- to put it at the front of divider 5 because the other pages are paginated, as indeed are these, but with a different system.

First of all, have you had an opportunity to read your notes?

LETBY: Yes.

JOHNSON: Do they mention blood in the nappy?

LETBY: No.

JOHNSON: I'm going to suggest to you that when you said just before the break that you had put it in your nursing notes, you knew that wasn't true.

LETBY: No, I couldn't recall my note specifically at that time.

JOHNSON: We'll come to those notes in a moment when we compare what you've put in the notes with other sources of information,

but I want to deal, as I have done in many other cases, with other people's descriptions of what they saw so that we can understand whether you agree or disagree that what they are describing is a true reflection of the presentation of this child.

LETBY: Okay.

JOHNSON: So starting with Dr Harkness, who gave evidence on 17 November last year. If anyone wants to check the references, they are between pages 38 and 41. So what I'm going to do is to read to you what these witnesses said about the discolouration that they saw. Do you understand?

LETBY: Yes.

JOHNSON: So this is Dr Harkness:

"So this was a strange pattern over the tummy area, over the abdomen, which didn't fit with the poor perfusion. In between the kind of chest/upper legs -- the rest was still pink but there were these kind of strange purple patches that appeared over the outside of his tummy."

Is that accurate or not?

LETBY: No, I don't agree that it was patches. It was purple but it wasn't patches.

JOHNSON: "From what I remember there were patches in one area, then there were some in the other. Some of it was still nice and pink, but it certainly was unusual and not fitting with a baby that had completely shut down with its -- or poor perfusion."

Would you say the same as you said before?

LETBY: Yes.

JOHNSON: "This was kind of a purply-blue colour."

Do you agree with that?

LETBY: It was purply-blue, yes.

JOHNSON: "So this was different in colour and different in pattern to what you would see from a perfusion perspective."

Do you agree with that?

LETBY: I don't think I can comment on that.

JOHNSON: "His head was pink, his upper legs, upper arms, chest was pink."

Is that accurate?

LETBY: I remember him to be more pale than pink.

JOHNSON: "The abdomen was purple, so patches -- so not one solid purple area. I'd say one purple patch to the bottom, one purple patch to the top, some in between."

Would you agree with that?

LETBY: No, I disagree.

JOHNSON: "I can't remember exactly where they were: I'd seen this in [Baby A] and that's the only other time prior to this that I had seen it."

Do you agree with that?

LETBY: No.

MR JUSTICE GOSS: Well, to be clear, what are you not agreeing with there? I think you have to break that down, Mr Johnson.

MR JOHNSON: Yes. The point I'm really looking for is: are you agreeing that, however you describe it, what you could see on [Baby E] was the same as what you had seen on [Baby A]?

LETBY: No, it was not the same.

MR JUSTICE GOSS: That's what I thought you meant, but I thought it was important to clarify that.

MR JOHNSON: What were the differences so we understand?

LETBY: With [Baby E], it was like a solid block of purpleness over his abdomen. [Baby A] had pale and whiteness and more of -- like a mottled look.

JOHNSON: He was then asked:

"Have you seen it since?"

And the doctor responded:

"Not outside of the babies in this case, no. They were different sizes. They weren't small dots, they would have been relative to the size of the baby, they were in the region of 1 to 2-

centimetre patches, possibly bigger but I wouldn't say smaller than that."

And do you agree with that?

LETBY: No.

JOHNSON: He then continued:

"The areas were over the abdomen, so not up on the chest, not below his groin, just in the middle section."

Do you agree that --

LETBY: Yes, I agree with that, yes.

JOHNSON: "From what I remember, it did change, where they were.

They were purple in one area and then they were purple in another."

LETBY: No, I don't agree with that.

JOHNSON: Right. Do you remember [Dr C] telling us that she didn't actually see what was on the discolouration that was on [Baby E] but Dr Harkness was animated when he described an unusual appearance to her?

LETBY: What do you mean by animated?

JOHNSON: Okay. That's his word. I'll try and put it in a more straightforward way. Do you remember that [Dr C] said that she didn't see discolouration, first of all?

LETBY: Yes.

JOHNSON: But do you remember that when she arrived to help, Dr Harkness had described to her what he had seen? Do you remember that?

LETBY: Yes.

JOHNSON: Were you there when Dr Harkness described to [Dr C] what he had seen?

LETBY: Not from my memory, no.

JOHNSON: Just so that I understand, because we have had misunderstandings before, are you saying that there was no description by Dr Harkness to [Dr C] or simply that you don't know whether he did describe it or didn't?

LETBY: I was not there for any conversation between the two of them.

JOHNSON: Okay. I'd like to go to your nursing notes, which do deal with this and do deal with other issues as well.

And rather than going back to ask you about specific parts, I'm going to ask you to read through what's in the notes. Can you see the typescript?

LETBY: Yes.

JOHNSON: It's not the best size, but if you have a problem just let us know and we'll put it on the screen and expand it for you. Could you read us through your nursing note that was created at 04.31 and completed at 04.51?

LETBY: "Written in retrospect for care given from 20.00. Emergency equipment checked. Fluids calculated. [Baby E] nursed in an incubator with humidity. IV fluids, Babiven and lipid via long line: 0.02ml per kilogram per hour of insulin via long line. Prior to 21.00 feed, 16ml mucky, slightly bile-stained aspirate obtained and discarded. Abdomen soft and non-distended. SHO informed. To omit feed. At 22.00, large vomit of fresh blood: 14ml fresh blood aspirate obtained from NG tube. Reg Harkness attended. Blood gas satisfactory. Blood sugar 10.7 millimoles. Metronidazole and IV ranitidine commenced and given. 10ml per kilogram of 0.9% sodium chloride bolus given. Mean BP and observations stable. [Baby E] handling well and active. NG tube on free drainage. Further 13ml obtained by 23.00."

JOHNSON: I think it says "13ml blood obtained", doesn't it?

LETBY: "13ml blood"2?

JOHNSON: Yes.

LETBY: Is that not what I said? Sorry, I apologise.

JOHNSON: I don't think you did but --

LETBY: I apologise:

"Further 13ml of blood obtained by 23.00. Beginning to desaturate and perfusion poor. Oxygen given via Neopuff, initially in 24% incubator oxygen. Toes becoming white and [Baby E] cool to touch. Reg Harkness present throughout. [Baby E] began to decline. At 23.40 became bradycardic. Purple band of discolouration over abdomen. Perfusion poor. CRT 3 seconds. Emergency intubation successful and placed on ventilator; see

medical notes. Required 100% oxygen. Saturations 80%. SIMV 22/5, rate 60. Further saline bolus and morphine bolus given. Second peripheral line sited and used to administer drugs. [Dr C] arrived.

"At 00.36 acute deterioration. Resus commenced as documented: x5 adrenaline, x2 sodium bicarbonate, x1 glucose bolus, x1 sodium chloride bolus, x1 20ml per kilogram O negative blood. At 01.01 chest compressions no longer required.

"Further decline and resus recommenced 01.15 and discontinued at 01.23 when [Baby E] was given to parents. [Baby E] was actively bleeding ++ from mouth and nose throughout the resus. 6ml blood was obtained from NG tube."

JOHNSON: Then "[Baby E] passed away" --

LETBY: "[Baby E] passed away at 01.40 in parents' arms."

JOHNSON: Now, for the sake of completeness, there is a further note that you made at 04.51 through to 04.58; is that right? We see that underneath.

LETBY: It's 05.37 on there.

MR JUSTICE GOSS: Yes, immediately below "[Baby E] passed away", the addendum.

MR JOHNSON: Do you see family communication? The bottom of the page.

LETBY: Sorry, I thought you were talking about the addendum.

JOHNSON: I'll come to that in a second, but just before we get to that. All that information that you produced in the first note, timed between 04.31 and 04.51, is that all carried in your head?

LETBY: No, it would have been written with reference to charts and any notes that had been made or medical notes.

JOHNSON: So whose notes -- first of all, had you -- other than the material that's in the fluid balance chart or the vital signs chart, had you written anything down contemporaneously at the time things were happening?

LETBY: That would be my usual practice, yes.

JOHNSON: What had you written down at the time things were happening?

LETBY: Well, I can't recall specifically now what I wrote down.

JOHNSON: Right. Well, it may be that we're at cross-purposes and I just want to clarify this. We have two primary sources of information. I think there should be down there with you the jury bundle full of charts with a 2 on the back of it if somebody could help you with that, please.

If you could go to divider 5, which is [Baby E]'s divider. At page 26284, which is the first page in that divider that you have in front of you, we have what I've called the vital signs chart. It's the observations chart, isn't it?

LETBY: Yes.

JOHNSON: So this is one source of contemporaneous information to you; is that right?

LETBY: Yes.

JOHNSON: So looking at the second page, which is 2685, we see that there are four columns which bear your initials?

LETBY: Yes.

JOHNSON: And that is material you were writing down at or about the time; is that right?

LETBY: Yes.

JOHNSON: Over the page again we've got blood gas at 2715, which has two entries, 22.21 on the 3rd and 01.05 on the 4th?

LETBY: Yes.

JOHNSON: We then have the fluid balance chart at 2717 and 2718, 2719 and 2720. Other than that material, was there any other notes that you were making at the time that would have gone into or been incorporated into the nursing notes?

LETBY: Yes, so the usual practice is when things are ongoing we write on the back of the handover sheet or on any piece of paper that's around at the time.

JOHNSON: Right. In your nursing notes, you describe the discolouration as a "purple band of discolouration".

LETBY: Yes.

JOHNSON: Don't you?

LETBY: Yes.

JOHNSON: Do you remember in your interview, and we can go to it if you would prefer, that you describe it as becoming "a patch of sort of purpleness"? Do you remember saying that?

LETBY: No, but I accept that if that's what that says.

JOHNSON: Well, I don't want you to simply agree with me. If you go to your interviews, please, it's in interview bundle 1, the [Baby E] one, the top of [Initials of Baby E] 7. It's the second reply of yours at the top of the page, which runs to four lines.

LETBY: Yes.

JOHNSON: "I noticed his abdomen was becoming fuller and rounder and later into the evening there was a discolouration area to part of his abdomen, like a purple discolouration."

LETBY: Yes.

JOHNSON: And over the page on page 8, what I described to you just before, just below halfway down the page:

"I noticed there was becoming a discolouration to his abdomen, but I can't say exactly where, but there was becoming a patch of sort of purpleness."

LETBY: Yes.

JOHNSON: And I think you gave further detail, didn't you, at the top of the next page, saying at the top of the page, [Initials of Baby E] 9:

"It was towards the right side, by his umbilicus, but I can't remember the extent or size at the moment."

LETBY: Yes.

JOHNSON: So at that stage, you were struggling to describe how big this patch was; is that fair?

LETBY: Yes.

JOHNSON: On Friday, 5 May, in answer to questions from your counsel, you described it as:

"There was a red sort of horizontal banding" --

LETBY: Yes.

JOHNSON: -- "across his abdomen."

LETBY: Yes.

JOHNSON: "Just across where his umbilical cord was, just around that area."

LETBY: Yes.

JOHNSON: And you were asked:

"Was it spreading anywhere else?"

And you said:

"No, it was just on the abdomen."

LETBY: Yes.

JOHNSON: Do you remember that?

LETBY: Yes.

JOHNSON: So does it come to this, that to a limited degree you and Dr Harkness agree in that it wasn't in the chest, it wasn't below the groin, it was across the abdomen?

LETBY: I agree that it was on the abdomen, yes. I do not agree that it was in patches.

JOHNSON: I would like to concentrate, if we can, please, on the paper charts because they're slightly easier to manipulate. If you put the interviews to one side for a second, please, and go back to bundle 2 that we were looking at before, which is where some of the jury have put the nursing note.

I'd like to go to the final page behind divider 5, please, which should have 2720 --

LETBY: Yes.

JOHNSON: -- in the bottom right-hand corner.

If we look along the bottom of the chart we can see that towards the right-hand quarter or so there are three columns bearing your initials.

LETBY: Yes.

JOHNSON: One which separates two sets of your initials, which are Belinda Simcock's initials --

LETBY: Yes.

JOHNSON: -- albeit some of the writing in that column is your writing, as I understood your evidence.

LETBY: Yes.

JOHNSON: So the "15ml fresh blood", for example, you told us was your writing, not Belinda Simcock's writing?

LETBY: Yes.

JOHNSON: I'll come to that in a moment. Can we just look at the aspirates line, please, which, if we count up from the signature line, is five lines above the signature line.

Have you got that?

LETBY: Yes.

JOHNSON: Do we see there were minimal aspirates, in effect, in the hours preceding [Baby E]'s collapse?

LETBY: Yes, that's right, yes.

JOHNSON: Would you agree that that is indicative of there being no gastrointestinal issue?

LETBY: Yes.

JOHNSON: The first hint of any gastrointestinal issue is your handwritten note in the 21.00 hours column, isn't it?

LETBY: Yes.

JOHNSON: That's the "16ml mucky"?

LETBY: Yes.

JOHNSON: That's your writing?

LETBY: Yes.

JOHNSON: The "omitted" above it is your writing?

LETBY: Yes.

JOHNSON: As is the data above the word "omitted"?

LETBY: Yes.

JOHNSON: In the 22.00 hours column, however, there is some of Belinda Simcock's writing?

LETBY: Yes.

JOHNSON: Why?

LETBY: I can't recall that right now. It's not unusual for nurses to carry out observations and readings for other people.

JOHNSON: Well, it isn't, but you apparently were there because you have written in to that column "15ml fresh blood".

LETBY: Yes. I'm assuming that that blood came after Belinda's documentation here.

JOHNSON: Why do you make that assumption?

LETBY: Because Belinda would have carried out those readings at 22.00 and it's my writing that says "Fresh blood", so I assume it's happened after Belinda has carried out those readings.

JOHNSON: But why was Belinda there at all?

LETBY: I can't -- I can't say for sure.

JOHNSON: Well, isn't it because you had drawn her attention to something?

LETBY: Yes, she did come to review the aspirate, yes, and she also assisted me with looking after [Baby F].

JOHNSON: Which aspirate are you talking about?

LETBY: The 16ml.

JOHNSON: That was an hour earlier, wasn't it?

LETBY: Yes.

JOHNSON: Yes, so what I'm asking you is why she was there at all.

LETBY: I can't answer that.

JOHNSON: If we go back to your nursing notes, which we've just handed out to the jury, we have the answer, don't we?

If you count up on the first page, count up four lines.

LETBY: Yes, 22.00.

JOHNSON: Yes.

LETBY: Yes.

JOHNSON: What does it say?

LETBY: "Large vomit of fresh blood."

JOHNSON: Yes. That's why Belinda was there, isn't it?

LETBY: No, I can't say why Belinda was there carrying out those observations at that time.

JOHNSON: "14ml fresh blood aspirate obtained from NG tube."

LETBY: Yes.

JOHNSON: Did that just appear or did it build up over a period of time?

LETBY: No, I believe that came into the free drainage pot.

JOHNSON: Yes, but over what period of time is the question?

LETBY: I can't be specific.

JOHNSON: Can't or won't?

LETBY: No, can't.

JOHNSON: You had injured [Baby E], hadn't you?

LETBY: No.

JOHNSON: And that's why he was bleeding?

LETBY: No.

JOHNSON: Why isn't there a reference on the paper chart to the vomit that you have recorded in the nursing notes?

LETBY: That's an error on my part.

JOHNSON: Well, it may be, but why?

LETBY: I can't explain that. Sometimes we don't document everything as accurately as we need to.

JOHNSON: Or is it in the excitement of sabotaging [Baby E], you overlooked it?

LETBY: No.

JOHNSON: Can we go to the -- sorry to have everybody jumping around between documents, but sometimes the answer is well hidden in this case. Can we go to the white neonatal review, please? Go to [Baby E], please, and could we go to line 59.

What does line 59 tell us, Lucy Letby?

LETBY: That at 22.00, Belinda gave a feed to JE.

MR JUSTICE GOSS: Hang on, can we just wait?

MR JOHNSON: Sorry.

MR JUSTICE GOSS: Some of you were tabbing it. It's in section 5, the fifth section. It is page 3 of 11 for [Baby E], near to the bottom. Have you all got that?

You see it now? Fine.

MR JOHNSON: A feed isn't the work of 30 seconds, is it?

LETBY: I can't comment on that without knowing what the volume of feed was.

JOHNSON: So Belinda Williamson was feeding a child in nursery 2 at 22.00 hours, wasn't she?

LETBY: Yes, approximately, yes.

JOHNSON: According to the paperwork?

LETBY: Yes.

JOHNSON: So my question, which I'm afraid is a repeat of the question I asked you earlier, is: why were you asking her to do observations on [Baby E]?

LETBY: I don't recall asking her to do that, so I can't answer that.

JOHNSON: Well, she was certainly busy at 22.00 hours, wasn't she, apparently?

LETBY: Yes, there were also other staff with her baby as well, yes.

JOHNSON: Was it so that at the time of his collapse, you had somebody else's writing and signature on the paperwork?

LETBY: No.

JOHNSON: Like Caroline Oakley with [Baby D], did you get her to write in information that you had derived on to a chart?

LETBY: No. That wouldn't happen. She has read those drip readings for himself. That's not something that I could have relayed to her.

JOHNSON: Well, do you remember Caroline Oakley saying that "the girls" had told her information that she then wrote on to [Baby D]'s chart?

LETBY: Yes, but that wasn't specific values, that was regarding what she'd had suctioned. It wasn't the specific values of the drip reading.

JOHNSON: That's what you do, though, isn't it? You get other people to write things in on charts to cover up what you were doing?

LETBY: No, that's not correct.

JOHNSON: I'm going to ask you about some of the things that [Mother of Babies E & F] said to see whether you accept them.

First of all, she said that her husband [Father of Babies E & F] left her with [Baby E], having skin-to-skin, at about 5 pm, 17.00 hours, on 3 August. Do you accept that?

LETBY: I wasn't there so I can't comment.

JOHNSON: Do you accept that that skin-to-skin ended at 18.30?

LETBY: Again, I wasn't there, so I couldn't comment.

JOHNSON: Well, I'll show you [Nurse B]'s note, which is J2588, please.

I think it's a note the family (inaudible: coughing), it may be in the addendum possibly. Well, it'll be in a note made after 18.30 because 18.30 is the time that the skin-to-skin ended.

(Pause)

I think that's the wrong one. This is a specific time. Well, I can deal with that a different way. It's all right.

Do you know who it was that rang [Mother of Babies E & F]?

LETBY: When are we talking?

JOHNSON: When [Baby E] collapsed.

LETBY: No. I believe somebody contacted the midwifery staff rather than [Mother of Babies E & F] directly.

JOHNSON: Yes, of course, you're right, but at whose behest would that have been done?

LETBY: I don't know what that means, sorry.

JOHNSON: Who would have asked somebody to ring the midwifery staff?

LETBY: It would have been a collective decision. Once [Baby E] had started to deteriorate we would have all -- our first priority would have been to contact mum.

JOHNSON: Do you accept that [Mother of Babies E & F] made a phone call at 21.11?

LETBY: I accept she made a phone call, yes.

JOHNSON: Do you accept what she and her husband said was said in that call?

LETBY: No.

JOHNSON: That she was very upset, you don't accept that?

LETBY: No.

JOHNSON: Both she and her husband said that; you do not accept it?

LETBY: No.

JOHNSON: She told her husband that [Baby E] was bleeding from his mouth; you don't accept that?

LETBY: No.

JOHNSON: She'd been told there was nothing to worry about and to go back to the ward; you don't accept that?

LETBY: No.

MR MYERS: My Lord, she can't say what took place in a phone call and in fact asking, "Do you accept that? Do you not accept that", is a roundabout way of saying, "Is she telling the truth? Is she not telling the truth?" In any other case if a witness said they weren't party to a phone call it wouldn't be right to then ask them questions about what did or didn't happen in that phone call and Ms Letby can't answer that. All that's happening is that she's being invited to make comments on what was or wasn't said for the same reasons that those questions have been asked so far up to now again.

MR JOHNSON: Do you accept that just before the final of [Baby E]'s collapses, [Dr C] and Dr Harkness went to the computer to review the X-rays?

LETBY: I can't recall that from my memory, no.

JOHNSON: You may say you can't recall it, but it was the evidence of [Dr C].

LETBY: Okay. I don't have the benefit of being able to see that evidence right now. It's hard to remember who has said what over the course of the trial.

JOHNSON: Right. But that was the moment, according to [Dr C], of [Baby E]'s final collapse. Do you remember that at about 00.25?

LETBY: I can't remember where [Dr C] was, no.

JOHNSON: Where is the computer in nursery 1?

LETBY: Behind the partition wall.

JOHNSON: Yes, the same point at which Mel Taylor was when [Baby A] collapsed?

LETBY: Yes.

JOHNSON: Yes. Out of view of where [Baby A] and [Baby E] were?

LETBY: No, I don't agree. It was in view of [Baby A]. It wasn't for [Baby E], but it was in view of [Baby A].

JOHNSON: Well, Mel Taylor didn't agree, did she?

LETBY: No.

JOHNSON: You killed [Baby E], didn't you?

LETBY: No.

JOHNSON: And you injected him with air?

LETBY: No.

JOHNSON: Just as you had done with other babies before?

LETBY: No.

JOHNSON: Why in the aftermath were you so obsessed with [Mother of Babies E & F]?

LETBY: I don't believe I was obsessed with [Mother of Babies E & F].

JOHNSON: Let's look at -- can you tell us why you were searching for her continually?

LETBY: I often thought of [Baby E] and [Baby F].

JOHNSON: Mm. And on that subject, can we go to the [Baby F] interviews, please, at page 23? It's the third interview.

LETBY: Which folder is that, sorry?

JOHNSON: It's the interviews folder 1, please. It's [Initials of Baby F] 23.

You see there you were asked in effect the same question, weren't you, that I've just asked you, in a slightly different way, at the top of the page?

LETBY: Yes.

JOHNSON: "You searched for [Mother of Babies E & F] nine times and [Father of Babies E & F] once, from August to December?"

LETBY: Yes.

JOHNSON: And two in January. You were asked why and you said:

"To see how [Baby F] was doing."

LETBY: Yes.

JOHNSON: Now, the first of -- let me just check that my information is correct. The first of those searches, if we go to the [Baby E] sequence of events, please, is tile 273. Do you see that?

LETBY: Yes.

JOHNSON: Where was [Baby F] on Thursday, 6 August at 19.58?

LETBY: He was on the neonatal unit.

JOHNSON: Yes. So was what you said to the police and to the jury true, that you were looking to see how [Baby F] was?

LETBY: Collectively, yes, this is one search -- I agree this one is not, but the others were after [Baby F] had left, yes.

JOHNSON: All right. Deal with this one then, one of the ten.

Why were you searching for [Mother of Babies E & F] on Thursday, 6 August at 19.58?

LETBY: She was on my mind and when I think of people, I often search for them.

JOHNSON: You were looking to see what reaction you had got from this grieving family, weren't you?

LETBY: No.

JOHNSON: Just as you were on Christmas Day?

LETBY: No.

JOHNSON: Of all days. Tile 306, please. Didn't you have better things to do at 23.26 on Christmas Day than search for [Mother of Babies E & F]?

LETBY: No, I often thought of [Baby E] and [Baby F].

JOHNSON: Because you'd killed one and tried to kill the other, hadn't you?

LETBY: No, because I always thought [Mother of Babies E & F] and I had a good relationship.

JOHNSON: She's not the sort of person that would make things up about you, is she?

LETBY: I can't answer that. That's for her to answer.

JOHNSON: Let's move on to [Baby G], please. [Baby G]. Perhaps it would be a good idea, having said that I would forget, just to put everything away behind divider 5, please. I've just been reminded, I had told Mr Myers this, but I'll come back to [Baby F] later because I'm going to deal with him with [Baby L] because they are the insulin cases. So this is the one occasion where I'm going to go out of sequence, okay? Do you understand?

LETBY: Yes.

JOHNSON: We'll leave [Baby F] until later, until we get to [Babies L & M], but he's not forgotten. We will deal with [Baby G].

You will remember, I'm sure, that there are three allegations against you in relation to [Baby G].

LETBY: Yes.

JOHNSON: Count 7, which is the projectile vomit on 7 September 2015 --

LETBY: Yes.

JOHNSON: -- which coincides with her 100th day of life.

LETBY: Yes.

JOHNSON: And then counts 8 and 9, which were both 21 September 2015, which coincides with what would have been [Baby G]'s due date, had she gone full term.

LETBY: Okay. I'm not aware that was the date, but yes.

JOHNSON: Well, what are you not aware of?

LETBY: That that was [Baby G]'s due date.

JOHNSON: Is that the first time you've ever heard that information?

LETBY: Well, I can't recall. I don't recall knowing that that date specifically was her due date, no.

JOHNSON: You just think about that.

(Pause)

Are you really saying that the first time you have become aware that 21 September was [Baby G]'s due date is when I have just told you?

LETBY: No, I'm saying that that date does not stand out as being [Baby G]'s due date.

JOHNSON: Right.

LETBY: It's the date of the events not, in my mind, [Baby G]'s due date.

JOHNSON: Did it stand out to you as being [Baby G]'s due date at the time?

LETBY: Not that I recall, no.

JOHNSON: Can we go to [Baby G] 2, please. It's the sequence. Tile 106. Can you read that out, please?

LETBY: "Due date today!"

JOHNSON: You'd really forgotten that?

LETBY: Not at the time, no. Right here and now, yes.

JOHNSON: Or is the reason you are avoiding it, as I suggest, because of the coincidence between the events, that they coincide with day 100 of life and [Baby G]'s due date?

LETBY: No, that's all it is, a coincidence.

JOHNSON: In evidence to your counsel you said:

"She was a very premature baby that we do not often care for at the Countess of Chester Hospital."

LETBY: Yes.

JOHNSON: Do you remember saying that?

LETBY: Yes.

JOHNSON: By the time [Baby G] got to the Countess of Chester Hospital, she was well within the normal, in inverted commas, parameters of babies carried for at the Countess of Chester, wasn't she?

LETBY: Yes.

JOHNSON: So why did you say that she was very premature and you don't often care for babies like that?

LETBY: Because a baby born -- that extreme prematurity comes with a number of additional health issues that a prem baby that has been born, say, at more than 27 weeks wouldn't necessarily have. They require a different level of care.

JOHNSON: (Inaudible).

LETBY: I don't agree because a baby born like [Baby G] was born at the viability and she comes with a lot more issues and complex health conditions than a baby born at a different gestation might come with.

JOHNSON: But by the time she arrived at Chester, [Baby G] was fine, though, wasn't she?

LETBY: No, I don't agree that she was fine, no. She had a number of ongoing issues.

JOHNSON: We'll come to it. Can we start with your defence statement, please? If you'd like to put the interviews away just to reduce the clutter.

Paragraph 85 is the beginning of your defence statement for [Baby G]; do you agree?

LETBY: Yes.

JOHNSON: And at 85 you say:

"I did nothing to hurt [Baby G]."

LETBY: Yes.

JOHNSON: "Many of the children we care for had health problems.

[Baby G] had complex needs. Concerns were raised on the unit as to her movements between different units and whether our unit could properly care for her."

LETBY: Yes.

JOHNSON: [Baby G] was due to go home by the time she had the incident on 7 September, wasn't she?

LETBY: No, I don't agree that she was ready for home, no.

JOHNSON: Well, do you remember which nursery she was?

LETBY: She was in nursery 4.

JOHNSON: Yes, and which babies are put into nursery 4?

LETBY: Special care babies.

JOHNSON: Yes. And which is the nursery you go into when you're about to go home?

LETBY: Nursery 4, but you could be in that nursery for a period of months. It doesn't mean that you're imminently about to go home. She was being still nasogastric tube fed.

JOHNSON: Are you exaggerating [Baby G]'s problems?

LETBY: No.

JOHNSON: Which were -- you have raised the issue of there being problems. Which are the particular issues that you say [Baby G] faced which made her exceptional as at 7 September 2015?

LETBY: Collectively she had a lot of ongoing issues that came from being born so extremely premature.

JOHNSON: Give us a selection of these ongoing --

LETBY: They often have difficulties with feeding. I know [Baby G] had had many blood transfusions, she'd had previous sepsis, she'd had problems with her lungs.

JOHNSON: But as at September 2015, what were [Baby G]'s problems that you say made her unusual?

LETBY: I'm not saying that she was unusual, but a baby like that needs a certain level of care and observation that another baby might not.

JOHNSON: Well, she was getting a nursery nurse in nursery 4. Are you saying --

LETBY: By that point, yes. I'm not saying at that point. I'm saying earlier on.

JOHNSON: Well, it must be my fault then. I must not have made myself clear. I will now try to make myself clear.

What are you saying were [Baby G]'s unusual problems as at the date she first collapsed? In other words, as at 7 September 2015.

LETBY: When she had the vomit?

JOHNSON: Yes.

LETBY: She was a baby that was on oxygen and she'd had feeding issues.

JOHNSON: When had she had these issues?

LETBY: Throughout her life.

JOHNSON: No, no. Please don't try to deflect the question. The question is: as at 7 September, what were the particular unusual problems that [Baby G] had?

LETBY: I can't answer that.

JOHNSON: Well, perhaps we can just start by looking at the charts that we've all got in paper form behind divider 7 and you can give us chapter and verse on what you say are these problems.

LETBY: Which folder is it?

JOHNSON: Jury bundle divider 2. Divider 7.

MR JUSTICE GOSS: Behind divider 7.

MR JOHNSON: If we look at the first page, we've got the position as at the 2nd, 3rd, 4th and 5 September.

LETBY: Yes.

JOHNSON: So days 95, 96, 97 and 98 of [Baby G]'s life.

LETBY: Yes.

JOHNSON: Is there anything unusual about that picture?

LETBY: No.

JOHNSON: Turn over the page. We get to day 99, which is 6 September. Anything unusual on 6 September?

LETBY: No.

JOHNSON: The problems started when she projectile vomited, don't they?

LETBY: Yes.

JOHNSON: Yes. Turn over to page 6962, please. That deals with events after the projectile vomit, doesn't it?

LETBY: Yes.

JOHNSON: So on again, please. 6971. In about the bottom third of the page we've got half a dozen lines dealing with the 1st to 7 September; is that right?

LETBY: Yes.

JOHNSON: The very final line is a blood gas at 04.26, which is after the projectile vomit?

LETBY: Yes.

JOHNSON: Which bears your signature?

LETBY: Yes.

JOHNSON: But so far as the other five lines are concerned, dealing with September, is there anything untoward about any of that?

LETBY: No.

JOHNSON: It looks from that, doesn't it, that [Baby G] was off oxygen on 5 September?

LETBY: From this, yes. I'd have to look at the observation chart to confirm that, but yes. I know she was back on it on the 7th.

JOHNSON: Of course she was because by that stage she had brain damage, didn't she?

LETBY: No, I don't agree. She's been on oxygen on the observation chart we've just looked at.

JOHNSON: Well, if I in any way have misled people, you help us.

LETBY: She hadn't been off oxygen at all, she's remained on oxygen on these charts at all times.

JOHNSON: At the beginning, is that what you're saying?

LETBY: From this date of the 2nd, she at no point has been off oxygen.

JOHNSON: Right. What do these -- we can see saturations.

LETBY: Yes.

JOHNSON: If we look at page 6959 --

LETBY: Yes.

JOHNSON: -- we see [Baby G]'s saturations?

LETBY: Yes.

JOHNSON: Then we see O2, don't we?

LETBY: Yes.

JOHNSON: And we see at the beginning of -- I'll just wait for Mr Murphy to put it on the screen and then we can all see what we're talking about.

So we see SaO₂, which is saturations?

LETBY: Yes.

JOHNSON: That's a reading taken off the probe; is that right?

LETBY: Yes.

JOHNSON: Then we see O₂, which is the fractional oxygen that's being administered to the child; is that right?

LETBY: Yes.

JOHNSON: We see that that line begins at 30 --

LETBY: Yes.

JOHNSON: -- on 2 September at 23.00 hours? It runs across and it says 0.05 or other similar types of figures further on?

LETBY: Yes.

JOHNSON: What do they tell us?

LETBY: At this point [Baby G] has been put on to nasal prong oxygen, so it's prongs that go up the nose, and that's the percentage -- well, the fraction of units of oxygen that she's receiving.

JOHNSON: How many oxygen does that actually equate to?

LETBY: I don't know what it means in percentage terms -- is that what you're asking?

JOHNSON: Yes.

LETBY: I couldn't say, I don't know.

JOHNSON: All right. We won't get hung up on that. Can we go to the feeding charts, please, which are at the end of this document -- end of this section, I beg your pardon.

It's 7012 and 7013.

I'm going to -- can we start with 7012? Are you on that page?

LETBY: Yes.

JOHNSON: Do we see, in the top half of the page, data for 5 September?

LETBY: Yes.

JOHNSON: And we see that [Baby G] was being given expressed breast milk with Gaviscon and fortifier?

LETBY: Yes.

JOHNSON: We see the amount?

LETBY: Yes.

JOHNSON: Forty-four to 45ml every 3 hours?

LETBY: Yes.

JOHNSON: And we see the route by which it was delivered?

LETBY: Yes.

JOHNSON: Either NGT or B for bottle; is that right?

LETBY: Yes.

JOHNSON: Would you agree that all the signs on the 5th are good?

LETBY: Yes.

JOHNSON: On the 6th, which is in the bottom half of the page, again, similar data?

LETBY: Yes.

JOHNSON: And again would you agree that all the signs are good?

LETBY: Yes.

JOHNSON: And in particular, at 23.00 hours, we see your friend [Nurse E]'s signature?

LETBY: Yes.

JOHNSON: We see that [Baby G] was given a bottle at 23.00 hours?

LETBY: Yes.

JOHNSON: And to feed a 45ml bottle to a baby of [Baby G]'s age, how long would that take approximately?

LETBY: Um... For a baby on oxygen, maybe up to half an hour.

JOHNSON: Yes. So not the sort of thing you do twice by mistake?

LETBY: No, and I don't think I've ever suggested that.

JOHNSON: I'm not saying you have suggested it, I'm just checking that you agree that it's not the sort of thing that happens twice by mistake.

LETBY: No.

JOHNSON: No. So by the time we get over the page, 7013, to the feed at 02.00 hours, we see that we have the feed that immediately preceded the vomit.

LETBY: Yes.

JOHNSON: Just looking at all that data together, do you agree that, so far as feeding was concerned, the picture shown by the data is a good one?

LETBY: Yes.

JOHNSON: Do you also agree that [Baby G]'s observations immediately preceding her collapse after 02.00 hours on the 7th are good?

LETBY: Before 2 o'clock?

JOHNSON: Before 2 o'clock.

LETBY: Yes.

JOHNSON: We heard as much, didn't we, from [Baby G]'s father, from Dr Ventress and from your friend [Nurse E]?

LETBY: Yes.

JOHNSON: Yes. And you do not dispute that?

LETBY: No.

JOHNSON: And you knew, didn't you, that this was day 100 of [Baby G]'s life?

LETBY: Yes.

JOHNSON: It was a big deal?

LETBY: Yes.

JOHNSON: And if we look at tile 276, please, at the start of the night, such a big deal was it that you'd all been sitting at the nursing desk making a banner for [Baby G] to celebrate?

LETBY: That's what we do for babies of 100 days, yes. It's a big milestone, yes.

JOHNSON: Particularly for one who, as you have already told us, was born at the margins of viability?

LETBY: Absolutely, yes.

JOHNSON: A very big deal indeed?

LETBY: Yes.

JOHNSON: Yes. Well, I'm going to move on to the population distribution, which I can certainly deal with before the break. Can we go to tile 47, please, in [Baby G] 1.

If we scroll down, please.

Here we see, starting with [Baby G], she is the object of this allegation, she was in [Baby G] 2 (sic) with your friend [Nurse E]?

LETBY: Yes.

JOHNSON: The nursery nurse Lisa Walker had two children in nursery 3?

LETBY: Yes.

JOHNSON: You had one child in nursery 1 --

LETBY: Yes.

JOHNSON: -- IH, and Ailsa Simpson had another, AC?

LETBY: Yes.

JOHNSON: And there were two other children whose location cannot be confirmed, one was with Christopher Booth, DB?

LETBY: Yes.

JOHNSON: The other, CC, was under the charge of your friend, [Nurse E]?

LETBY: Yes.

JOHNSON: Any issue with staffing levels on this particular shift?

LETBY: Not from what I can see there, no.

JOHNSON: No. Are you suggesting that medical or nursing incompetence in any way has contributed to [Baby G]'s projectile vomiting on 7 September?

LETBY: No.

JOHNSON: Are you suggesting that in relation to the treatment of [Baby G] up to the point at which she projectile vomited on 7 September, anybody, medical or nursing, made a mistake?

LETBY: Potentially nursing, yes, but I'm not saying that did happen.

JOHNSON: What's the potential problem that you have identified?

LETBY: That [Baby G] received more than the 45ml at 2 am.

JOHNSON: So your friend, [Nurse E], may have overfed [Baby G]; is that what you're saying?

LETBY: That's a possibility. I'm not saying that's what she did, but that's what's been concluded, those are the options: that or she hadn't digested the milk that was in her prior to that feed.

JOHNSON: So does it come to this, that because it's an alternative to you having injected milk into [Baby G], you are not ruling out the possibility that your friend, one of your best friends, [Nurse E], overfed her?

LETBY: I can't say for definite that that didn't happen, no.

I'm not saying she did do that, but it's a possibility.

JOHNSON: Just in the 2 minutes we've got before the break, you tell us what would have had to have happened for [Nurse E] to have overfed [Baby G].

LETBY: I don't believe that she would have, but potentially she has mismeasured the amount of millilitres when she's decanted the milk from the bottle into the feeding bottle.

JOHNSON: We know, because you have told us, that the syringe is a 10ml syringe?

LETBY: Yes.

JOHNSON: So not only has she miscalculated the original amount, but she's also put in or allowed to drip feed in more than 4.5 measures of 10ml? So it's a double mistake?

LETBY: That was a possibility, yes.

JOHNSON: Well, everything is possible, like Dr Marnerides' helicopter and the pots, but are you suggesting it's a realistic possibility?

LETBY: No.

MR JOHNSON: No. Well, my Lord, that may be a good moment.

MR JUSTICE GOSS: Yes, certainly.

We'll break off until 2.10 then, please, members of the jury. Remember your responsibilities as jurors.

(1.00 pm)

(The short adjournment)

(2.10 pm)

MR JOHNSON: Just before the break I was asking you questions about the possibility of your friend, [Nurse E], having overfed [Baby G].

LETBY: Yes.

JOHNSON: And you accepted that it wasn't a realistic possibility; is that right?

LETBY: It's a possibility. I don't believe that happened, no.

JOHNSON: We know from the paperwork what [Nurse E] recorded as having fed [Baby G], don't we?

LETBY: Yes.

JOHNSON: We can see it's behind divider 7 at the very final page, J7013, which you have open in front of you.

LETBY: Yes.

JOHNSON: To have fed [Baby G] twice as much presumably would have taken twice as long?

LETBY: Yes.

JOHNSON: To have fed 90, taking it twice, 90ml at 2 o'clock, how long would that take?

LETBY: I can't say for definite, but it would have taken longer than the 45ml, yes.

JOHNSON: How long does 45ml take?

LETBY: It's very hard to say definitively, but 15/20 minutes, maybe.

JOHNSON: And do you remember what time you recorded the desaturation as having occurred at?

LETBY: Yes, 2.15.

JOHNSON: Yes. So on those grounds as well, we can exclude the possibility that [Nurse E] overfed [Baby G], can't we?

LETBY: I believe the evidence presented was that overfeeding is a possibility.

JOHNSON: All right. Who presented that evidence?

LETBY: Is that not what the experts have said?

JOHNSON: How would the experts know? They weren't there. Which experts said overfeeding was a possibility? (Pause).

Can you help us?

LETBY: Have Dr Evans and Dr Bohin not made reference to overfeeding?

JOHNSON: Do you remember saying in answer to your counsel that you remembered being with Ailsa Simpson just prior to [Baby G] vomiting?

LETBY: Yes.

JOHNSON: That you were sitting together at the nursing station?

LETBY: Yes.

JOHNSON: You'd been there for a few minutes?

LETBY: Yes.

JOHNSON: You heard an alarm and quite a loud retching noise?

LETBY: Yes.

JOHNSON: And so you both went to [Baby G] in nursery 2?

LETBY: Yes.

JOHNSON: When you went in, you found her vomiting from the mouth and the nose?

LETBY: Yes.

JOHNSON: So does that mean she was still vomiting by the time you got there?

LETBY: Yes.

JOHNSON: You started giving her Neopuff?

LETBY: Yes.

JOHNSON: Stripped her off and found her abdomen to be firm and distended and red?

LETBY: Yes.

JOHNSON: You were both quite shocked, is what you said?

LETBY: Yes.

JOHNSON: There was vomit on the chair at the cot side and on the floor?

LETBY: Yes.

JOHNSON: And that shocked you as well?

LETBY: Yes.

JOHNSON: Neither of you had ever seen that before?

LETBY: No, not on a neonatal unit, no.

JOHNSON: And that was a clear recollection that you had last week, giving evidence?

LETBY: Yes.

JOHNSON: What time do you say it was that [Baby G] vomited?

LETBY: 02.15 approximately.

JOHNSON: Let's see what you wrote in the sequence. It's at tile 79, please:

"[Baby G] had a large projectile milky vomit at 02.15."

LETBY: Yes.

JOHNSON: "Continued to vomit ++"; yes?

LETBY: Yes.

JOHNSON: "45ml milk obtained from NG tube with air ++"?

LETBY: Yes.

JOHNSON: "Abdomen noted to be distended and discoloured. Colour improved a few minutes after aspirating tube. Remained distended but soft. Registrar Ventress asked to review: to go nil by mouth with IV fluids. Doctor called to theatre."

And that's a reference, isn't it, to Dr Ventress being called away to a more pressing emergency?

LETBY: Yes.

JOHNSON: If [Baby G] had not digested the feed given at 23.00 the previous night, [Nurse E] wouldn't have got an acidic reaction, would she, when she --

LETBY: Yes, she would, yes.

JOHNSON: -- tested the nasogastric tube?

LETBY: Yes, she would.

JOHNSON: So you disagree with Dr Bohin?

LETBY: Yes, I do.

JOHNSON: On what basis do you disagree?

LETBY: Because having looked after many babies over the years, we can get milk aspirates with a positive acid reaction.

JOHNSON: What, a pH of 4?

LETBY: Yes.

JOHNSON: It's very acidic, isn't it?

LETBY: Yes.

JOHNSON: [Baby G]'s stomach wouldn't have accommodated a further 45ml of feed, would it, under gravity if she had still been full after the feed at 23.00?

LETBY: I think that's a medical thing. I can't comment on the size of a baby's stomach.

JOHNSON: Can we look at the photograph that Ailsa Simpson produced, please? It's AS4, which is J26510.

Do you agree that is accurate?

LETBY: Yes.

JOHNSON: I think you told us, in answer to your counsel, that this was something that you had never seen before.

LETBY: Not in a neonate, no.

JOHNSON: Do you remember what you told the police?

LETBY: Yes.

JOHNSON: What did you say about the spread of it to the police?

LETBY: That it was in the cot and down her clothing.

JOHNSON: Yes. If we just go to your interview, please. It's the first [Baby G] interview, so it's still in the interviews bundle. Page 6, please. [Initials of Baby G] 6. At the bottom of that page, you describe...

"When you've called it a large vomit, can you describe -- where did it go? What colour was it?"

And your response, please?

LETBY: "From what I remember, it was milky coloured vomit, but it was a large volume that was going into the bed and down [Baby G]'s clothes."

JOHNSON: Why didn't you tell the truth about this?

LETBY: I haven't lied. It was in her cot. I just at that moment haven't specified exactly where else it went.

JOHNSON: No, but this was an extraordinary vomit, wasn't it?

LETBY: Yes.

JOHNSON: The like of which you had never seen in your career before or --

LETBY: I have seen but not on a neonate.

JOHNSON: Yes. It would have stuck in your mind, wouldn't it?

LETBY: Yes.

JOHNSON: So why didn't you tell the truth to the police?

LETBY: It's not telling the truth, it's an oversight that I haven't mentioned it during interview.

JOHNSON: Did you remember it when you were saying this to the police?

LETBY: I can't say that now.

JOHNSON: Where were you when you saw [Baby G] vomit?

LETBY: [Baby G] was vomiting when Ailsa and I went into the nursery to her. She was still vomiting.

JOHNSON: Where were you?

LETBY: With Ailsa at the cot side of [Baby G].

JOHNSON: You were not with Ailsa, were you?

LETBY: Yes, I was.

JOHNSON: If we look at what you said to the police back in July 2018. So moving on from page 7 where we've just been on to page 8. Almost exactly halfway down:

"Was that the first sign to you that something was wrong with [Baby G]?"

Okay?

LETBY: Yes.

JOHNSON: And this is after you've told the police on the previous page that you've gone in. You say:

"Answer: I think so, yes. She'd been a baby that had been backwards and forwards to different units at other times.

"Question: And you know when you said air + from the NG tube, how do you measure that amount of air? How did you come to that conclusion?

"Answer: So when I've withdrawn the 45ml of milk, air came out with it.

"Question: What determined, to your mind, to put ++ on your notes?

"Answer: It was a large amount. I haven't measured the volume."

You then repeat what you had said on the previous page about where the vomit was; is that right?

LETBY: Yes.

JOHNSON: And over the page, about halfway down -- about a third of the way down they say:

"So at the point of the vomit what exactly were you doing?"

And you say:

"I don't remember."

LETBY: That's right.

JOHNSON: So you didn't know where you were at the time of the vomit?

LETBY: Not at this point, no.

JOHNSON: But you were present at [Baby G]'s cot and you say:

"I think I went to her because she was vomiting, I heard her vomiting."

LETBY: Yes.

JOHNSON: Is there any mention here of being with Ailsa?

LETBY: No.

JOHNSON: Why not?

LETBY: I've just told you my action, that I went in to her.

JOHNSON: You weren't with Ailsa, were you?

LETBY: Yes.

JOHNSON: Let's look at the neonatal review if we may, please. If you just put that away, if you wouldn't mind, and go to the white neonatal review file. If you go to the first [Baby G] -- it may be number 7, it's the first [Baby G] neonatal review. Go to page 2 of 7. We see the layout that we dealt with just before the break.

Got that?

LETBY: Yes.

JOHNSON: Can we go to page 3 of 7, please. What we see at page 3 may be slightly misleading because at line 37 it says "[Baby G] event".

LETBY: Yes.

JOHNSON: And it's been marked in there at 2 o'clock.

LETBY: Yes.

JOHNSON: On your evidence and your note made at the time it was 2.15, wasn't it?

LETBY: Yes.

JOHNSON: And so would be at the same point as line 40?

LETBY: Yes.

JOHNSON: First of all, I'm going to suggest to you that you deliberately misstated the time at which [Baby G] had this event, had this vomit --

LETBY: No.

JOHNSON: -- and that it happened much closer to 2.30.

LETBY: No, I don't agree.

JOHNSON: Right. We'll come to the reasons for that in a moment because I'm suggesting that this is one of the occasions where you have misrepresented events in the nursing note. All right?

LETBY: Okay.

JOHNSON: Before we get there, let's look at what was going on with Ailsa Simpson. What we see in this neonatal review at line 41 is that she was feeding the child AC --

LETBY: Yes.

JOHNSON: -- is that right, at 2.20?

LETBY: Yes.

JOHNSON: If we go back to page 2, we see that the child AC was in nursery 1?

LETBY: Yes.

JOHNSON: You will remember evidence that Ailsa Simpson gave us on 2 December last year when she said -- she described to us the routine that she would have had to have undertaken to fetch the milk, to defrost it, to warm it and to feed it?

LETBY: Yes.

JOHNSON: And if she fed this child at 2.20 she was certainly not with you at the nursing desk at the time [Baby G] collapsed, was she?

LETBY: I disagree. We were together at the nurses' station.

JOHNSON: Well, let's just look at, in terms of the time, Dr Ventress' note at tile 80, please. This is line 44 in the neonatal review:

"Called to review [Baby G] urgently at 02.35."

LETBY: Yes.

JOHNSON: That's when she was called, isn't it?

LETBY: From her documentation. I can't confirm exactly when she was called, I'd have to look at the bleep records.

JOHNSON: That is an accurate time, I'm going to suggest, for when this event actually happened.

LETBY: I disagree.

JOHNSON: And I am suggesting to you that this is the evidence that shows that you have deliberately put the time of the vomit forward to 2.15.

LETBY: No, I disagree.

JOHNSON: And the reason you've done that, I'm going to suggest, is so that there is a more obvious link between your friend, your best friend, [Nurse E], feeding the child and the vomit.

LETBY: No.

JOHNSON: Because you wanted to create the impression that the two events were linked.

LETBY: No.

JOHNSON: Whereas in fact, the reason [Baby G] collapsed and the reason you aspirated so much air and milk from her is that you deliberately overfed her.

LETBY: That's not true.

JOHNSON: And you did it by putting milk into a syringe and using the plunger to force air and milk into her.

LETBY: No, that's not true.

JOHNSON: Where did all that air come from?

LETBY: She was Neopuffed.

JOHNSON: But you extricated, you aspirated the air before she was Neopuffed?

LETBY: I can't say that without looking -- I can't confirm or deny that at the moment. I'd have to look at the charts.

JOHNSON: Which chart are we going to look at to find the answer to that point?

LETBY: So usually, as we went straight to [Baby G] and started Neopuffs, that would have been -- the first priority would have been her breathing rather than aspirating a tube.

JOHNSON: Well, let's look at your note, shall we, tile 79? Can we go to the original, please, in case there's a disconnect between what we have here and what's in the original?

Bottom right-hand quadrant. This is your note; is that right?

LETBY: Yes.

JOHNSON: Written at 08.57 and concluded at 09.11?

LETBY: Yes.

JOHNSON: Was there any other information that you had written down at the time which underlied this?

LETBY: What do you mean, sorry?

JOHNSON: Had you written down a different account somewhere else before you --

LETBY: Not a different account, no.

JOHNSON: Okay. So can we take it that this is an accurate account of what happened?

LETBY: Yes, as much as it can be accurate, yes.

JOHNSON: Subject to the time, you say:

"Written in retrospect for care given from 02.00. [Baby G] had large projectile vomit at 02.15. Continued to vomit. 45ml milk obtained from NG tube with air ++. Abdomen noted to be distended and discoloured. Colour improved few minutes after aspirating tube."

LETBY: Yes.

JOHNSON: Where does it mention Neopuffing?

LETBY: It doesn't.

JOHNSON: Why not?

LETBY: That's an oversight. Ailsa and I both said in interview that she needed Neopuffing.

JOHNSON: Neopuffing doesn't account for all this air in [Baby G], does it?

LETBY: That's not for me to answer.

JOHNSON: Does milk come out of the Neopuff machine?

LETBY: No.

JOHNSON: No. It doesn't account for all the milk either, does it?

LETBY: No.

JOHNSON: Because the truth is that you injected [Baby G] with milk and air, didn't you?

LETBY: No.

JOHNSON: Can we go to page 20 of your interview, please? This is the second [Baby G] interview. This is the interview of 11 June 2019. It's page [Initials of Baby G] 20, if anyone is still looking, please. You were asked all these questions by the police, weren't you?

LETBY: Yes.

JOHNSON: You see -- we'll start halfway down the page:

"Did you administer air to [Baby G] at this time causing her to collapse?"

Do you see that; yes?

LETBY: Yes.

JOHNSON: "Answer: No.

"Question: In your previous interview, Lucy, you said you obtained a larger volume of air from the NG tube than just the general amount you would usually obtain.

"Answer: Yeah. I'm not sure. At what point did I aspirate the tube and find the air? Was it when she vomited?"

So this is what I've just been asking you about.

LETBY: Yes.

JOHNSON: The police say:

"Question: Yeah, you said after a vomit you took over her care as the designated nurse and you obtained a larger volume of air from the NG tube than just the general amount you obtained.

"Answer: Yes.

"Question: Would you agree that she must have received a bolus of air from the feeding syringe?"

And what was your answer?

LETBY: "Well, air has got there somehow, yes".

JOHNSON: And that's the truth of it, isn't it?

LETBY: No. It's been suggested to me that it could have gone through the feeding syringe and I've said potentially.

JOHNSON: And how would it potentially get through the feeding syringe?

LETBY: If it's been put down there.

JOHNSON: How would you do that?

LETBY: Pushing it down.

JOHNSON: With what?

LETBY: The plunger.

JOHNSON: Yes. That's what you were thinking, weren't you?

LETBY: No.

JOHNSON: And that's why you gave that answer?

LETBY: No, that's how it was put to me.

JOHNSON: Where do they say with a plunger?

LETBY: They don't, but they've suggested that the air came from the NG tube to me.

JOHNSON: About 45 minutes later, at 03.15, do you remember Dr Ventress telling us that she was called out of theatre because [Baby G] had collapsed again?

LETBY: Yes.

JOHNSON: And this was when you were her designated nurse, wasn't it?

LETBY: Yes.

JOHNSON: Do you remember what Dr Ventress saw in [Baby G]'s mouth when she tried to intubate her?

LETBY: Not from my memory, no.

JOHNSON: Well, I will remind you: bloodstained fluid coming up from the trachea between the vocal cords.

LETBY: Okay.

JOHNSON: So echoes of [Baby C] and [Baby E]; would you agree with that?

LETBY: I don't recall them specifically saying it came up through the cords on [Baby E], but...

JOHNSON: It was coming up from somewhere down there, wasn't it?

LETBY: Yes.

JOHNSON: Yes. When you had been alone with [Baby G]?

LETBY: I'm not saying that I've been alone with [Baby G].

JOHNSON: And you had inserted something into [Baby G]'s airway, hadn't you?

LETBY: No.

JOHNSON: And you caused the bleeding, as you did with many other of these children?

LETBY: No, that's not true.

JOHNSON: Do you remember, later [Baby G] desaturated on the ventilator at 5.30 and again at 6 o'clock?

LETBY: Yes, I do, yes.

JOHNSON: And 100ml of either air or milk was aspirated; do you remember that?

LETBY: 100ml?

JOHNSON: 100ml.

LETBY: Can I see where that's documented, please?

JOHNSON: Yes, it was the evidence given by Dr Ventress and by -- Dr Ventress said air, Dr Brearey said milk.

LETBY: Is it on the charts though?

JOHNSON: It's the evidence that was given.

LETBY: But is it documented?

JOHNSON: No, no, I'm telling you what the evidence is; do you agree with it or not?

LETBY: I don't recall 100ml coming out, no.

JOHNSON: You put that there, didn't you?

LETBY: 100ml?

JOHNSON: Yes.

LETBY: No.

JOHNSON: Well, how much did you put there?

LETBY: I didn't put any air down.

JOHNSON: As soon as you went off shift, do you remember who took over [Baby G]?

LETBY: [Nurse A].

JOHNSON: Yes. Do you remember what problems [Nurse A] had with [Baby G] after you had disappeared?

LETBY: No.

JOHNSON: Let's look at tile 320, please -- actually, sorry, can I just -- we'll come to that in a second. Can we just go back to tile 118, please? This is Dr Ventress' note.

I've found the documentation you just asked me for.

We'll go to Dr Ventress' handwriting in a second, just to check that this is a true reflection of what she has written. But do you see:

"ETT removed at 06.10, thick secretions ++"?

LETBY: Yes.

JOHNSON: "Blood clot at end of ETT."

LETBY: Yes.

JOHNSON: Then the last two lines:

"NG aspirated as abdo appeared very large. About 100ml aspirated."

LETBY: Yes.

JOHNSON: So that's documented.

LETBY: No, I agree it's documented, I just don't recall that volume.

JOHNSON: Do you accept it?

LETBY: I accept it if that's what they've written there, yes.

JOHNSON: Yes. How did that get there? Could you speak up, please?

LETBY: I don't know how the air got there. It's after Neopuffing.

JOHNSON: Let's go back to the point I was making before I managed to locate that note. It's tile 320, please. Can we go to the original note, please? Sorry.

That's on the left, please, if we can expand that.

Thank you.

So this is your friend [Nurse A]'s note, isn't it?

LETBY: Yes.

JOHNSON: No suggestion there that there was any build-up of either fluid or air in [Baby G], is there?

LETBY: Not that I can see there, no.

JOHNSON: No. Do you remember what you were doing that night when you were not in work?

LETBY: I went back into work to sign for some documentation for [Baby G].

JOHNSON: You went to visit [Baby G] didn't you?

LETBY: I didn't visit [Baby G], no. I went to do what I needed to do with documentation.

JOHNSON: Were you looking for an opportunity to finish her off?

LETBY: No.

JOHNSON: Let's move on to count 8, if we may, please, which is 21 September 2015. This is the day that [Baby G] would have been full term had she remained in utero; do you remember?

LETBY: Yes.

JOHNSON: I want to start, if we can, with your defence statement, please. If we can go to paragraph 92. Okay?

LETBY: Yes.

JOHNSON: What you say here is:

"On 21 September 2015, [Baby G] vomited at about 10.15. This was not as pronounced as the vomit on 7 September. I think her mother was there."

What made you think her mother was there?

LETBY: Because I'd recalled, and I'd written in my notes, that mum was visiting around 10 o'clock that morning.

JOHNSON: "The doctors were on the unit so I went to the nurses' station to ask the doctors who were doing the ward rounds to come and see [Baby G]. I did not shout for help because I did not think this was an emergency."

LETBY: Yes.

JOHNSON: Is that true?

LETBY: Yes.

JOHNSON: "I do not recall her [that is [Baby G]] going blue."

LETBY: No.

JOHNSON: You had seen [Baby G] many times on the unit, hadn't you?

LETBY: Yes.

JOHNSON: Do you agree with her father that after that first incident on 7 September, she was not the same baby?

LETBY: I can't comment on that. Nobody knows their child like the parents do.

JOHNSON: If we could look at tile 44 of the second sequence for [Baby G], please. My reference must be wrong here, so I'll just check it. Tile 43, please.

So let's just look at what we have here. Is that Val Thomas whose signature is at the top?

LETBY: Yes.

JOHNSON: So she's a band 4; is that right?

LETBY: Yes.

JOHNSON: [Baby G], I think, was by this stage in nursery 4?

LETBY: She was, yes.

JOHNSON: We see that at 00.15 and 03.15 and 06.00, Val Thomas fed [Baby G], partially by bottle and partially by nasogastric tube?

LETBY: Yes.

JOHNSON: And the amount of milk was alternating between 45 and 40ml each time?

LETBY: Yes.

JOHNSON: Why were you feeding her with a nasogastric tube at 9.15?

LETBY: She wasn't awake and she was due her immunisations.

JOHNSON: What inferences do we draw from the fact that [Baby G] was about to have her immunisations?

LETBY: That she was well enough to be receiving them.

JOHNSON: Yes. Was that a decision with which you agreed or disagreed from what you had seen?

LETBY: Up until the point of the vomit, yes, I agreed with that.

JOHNSON: Yes. Well, if you hadn't, presumably, like you say you did with, was it [Baby N] -- no, [Baby Q], I beg his pardon, you wouldn't have fed him and you would have waited for the doctors, wouldn't you?

LETBY: Yes.

JOHNSON: So can we work on the basis that when you took over [Baby G]'s care that morning, she was in a good condition?

LETBY: I remember I had some concerns about her observations, her temperature, but feeding-wise yes.

JOHNSON: And what was your concern about her temperature?

LETBY: I think she had an ongoing low temperature. I can't remember specifically, I'd have to check the chart.

JOHNSON: Well, we will come to it, okay? I just wanted to know what you said first of all.

Before we do, though, can we go to the population distribution chart at tile 45, please. Here we see that you had three children.

LETBY: Yes.

JOHNSON: All were in nursery 3?

LETBY: 4.

JOHNSON: 4, I beg your pardon.

LETBY: Yes.

JOHNSON: That Janet Cox, who was a nursery nurse, was working in nurseries 3 and 2?

LETBY: Yes.

JOHNSON: Did that cause you a problem, that someone of lesser nursing status was in a more elevated nursery?

LETBY: No, because it doesn't necessarily reflect the care needs of the baby, depending on what nursery they're in.

JOHNSON: Just previously you've suggested that whenever a child is in nursery 1, it means they're ITU, but that isn't right, is it?

LETBY: Not every time, no.

JOHNSON: Did it annoy you that you were in nursery 4?

LETBY: Not at all, no.

JOHNSON: One of the things that you said in answer to your own counsel was that aspirating from a child's nasogastric tube can interrupt the digestion.

LETBY: If you fully withdraw a large amount, yes.

JOHNSON: Wouldn't it have made more sense, given that [Baby G] was now a full term baby and this was 9.15 in the morning, to have fed her with a bottle?

LETBY: No, [Baby G] was not awake, she was having alternate bottle and tube feeds and she was due her immunisations, so as

part of that process we rest the baby because they can often become unwell after immunisations.

JOHNSON: I want to just check that I understood what you said in answer to questions of your own counsel; okay?

LETBY: Okay.

JOHNSON: You told your own counsel that you remembered giving a feed to [Baby G] at 09.00 hours.

LETBY: Yes.

JOHNSON: And you were then shown tile 47, so if we can go to tile 47. This then, I think, jogged your memory that in fact it was 9.15.

LETBY: Yes.

JOHNSON: Is that right?

LETBY: Yes.

JOHNSON: You then said that the comments after that, so you were looking at this document that we see, which is J7883, the comments after that followed the feed. That was what you said.

LETBY: Yes.

JOHNSON: By that, you were referring to, "30ml x2 milk projectile vomit"; is that right?

LETBY: Yes, all of the writing after that, yes.

JOHNSON: I just want to check that we all understand what the writing is, first. After 30ml there's something in a circle. What does that say?

LETBY: That's a D for discarded.

JOHNSON: Is it 4.2 or 9.2?

LETBY: 9.2.

JOHNSON: What does that refer to?

LETBY: The blood sugar. It's in the BM column, which is blood sugar.

JOHNSON: Then "Bowels opened +++"?

LETBY: Yes.

JOHNSON: "Loose and watery, green?"

LETBY: Yes.

JOHNSON: Then "Reviewed by doctors"?

LETBY: Yes.

JOHNSON: Which doctors are we talking about?

LETBY: I can't recall specifically, I think it was Dr Fielding.

JOHNSON: So reviewed by a doctor, not doctors?

LETBY: Well, I can't say whether there is one or more than one.

The ward round is done by more than one doctor. I can't be specific.

JOHNSON: You then said:

"I cannot be sure if mum was present at this time."

LETBY: Mm.

JOHNSON: You've referred to a low temperature, haven't you?

I think you did it a --

LETBY: I think it was low. There was something about her observations, I think, yes.

JOHNSON: There was. Tile 46, please. If we make it slightly smaller, we'll get everything on to a single screen.

Do you see 9 o'clock?

LETBY: Yes.

JOHNSON: If we scroll down a bit so we keep 9 o'clock at the top, please, Mr Murphy.

Look down the 9 o'clock column. What do we see in the temperature readings?

LETBY: The temperature within the white zone.

JOHNSON: It's a bit like doctor and doctors: is it temperature or temperatures?

LETBY: I don't understand what you mean.

JOHNSON: Do you really not?

LETBY: No.

JOHNSON: What do you see in that 9 o'clock column against temperature? How many readings have been recorded?

LETBY: One. I believe that other -- I don't know what that is but the usual way we write a temperature is a dot followed by a left arrow, which is what I've documented there.

JOHNSON: There are two dots in that column, aren't there?

LETBY: There appears to be something, yes.

JOHNSON: If it isn't a dot, what are you suggesting it is, just so we can bear it in mind?

LETBY: I don't believe that's me writing a temperature down.

That could just be an error or -- I can't explain that.

The normal way we write a temperature is to do a dot and then the line coming from it.

JOHNSON: There's a temperature that's very close to normal, isn't there?

LETBY: Yes. They're both normal if that --

JOHNSON: Yes (overspeaking). But the upper one, which you appear to be disavowing, is exactly the same as was recorded by a different nurse at 06.00, isn't it?

LETBY: Yes.

JOHNSON: Did you go back after the event and cook the chart to make it look like [Baby G] was declining before she vomited?

LETBY: No.

JOHNSON: You understand what I'm suggesting to you, don't you?

LETBY: Yes, [Baby G] had a low temperature preceding the shift, that's not normal to have a continuing low temperature.

The one value that I took is not what I was talking about, I was talking about -- the general pattern is that she had low temperature.

JOHNSON: We know what you said but I'm suggesting you have altered this chart --

LETBY: I haven't, no.

JOHNSON: -- to make the position look as though [Baby G] was declining before she vomited.

LETBY: No, if that was her temperature, both of those dots are within the normal range.

JOHNSON: Oh yes, but the pattern -- we have heard from many doctors, haven't we, that you need to look at the way the pattern goes --

LETBY: Yes.

JOHNSON: -- you need to look at the context. And we can see the one you recorded at 11 o'clock after [Baby G] had vomited and declined, can't we?

LETBY: Yes.

JOHNSON: And the lower one of the two that you have recorded at 9 o'clock is pretty much on a straight line between the 6 o'clock and the 11 o'clock one, isn't it?

LETBY: Yes.

JOHNSON: Whereas the other one is not.

LETBY: 3 o'clock, is it? The 3 o'clock. Sorry, I can't...

JOHNSON: Don't worry.

LETBY: I think it's the 3 o'clock, it's in the yellow, and that's the same, yes, at 11.00.

JOHNSON: We've got two in the 3 o'clock as it happens. Anyway, at 6 o'clock, we've got one that's up close to the normal line, which is 37, is it, or 36.8?

LETBY: Potentially somebody else has misdocumented two temperatures then.

JOHNSON: Yes, absolutely. I am more interested in what you have misdocumented.

LETBY: I haven't misdocumented anything.

JOHNSON: The projectile vomit you recorded at 10.15; is that right?

LETBY: Yes.

JOHNSON: Can you see tile 50, please?

"At 10.15, two large projectile milky vomits."

LETBY: Yes.

JOHNSON: Then you record:

"Brief self-resolving apnoea and desaturation to 35%..."

LETBY: Yes.

JOHNSON: Which we have heard is a life-threatening level potentially.

LETBY: Yes.

JOHNSON: "... with colour loss. NG tube aspirated. 30ml of undigested milk discarded."

This is 30 of the 40 or 45 that had been given at 9.15?

LETBY: Yes.

JOHNSON: Plus two large projectile vomits?

LETBY: Yes.

JOHNSON: And despite all that and despite the vomiting, the abdomen was still distended?

LETBY: Yes.

JOHNSON: It's not an innocent coincidence, is it, that [Baby G] vomited and had to have a lot of milk removed from her just after or an hour or so after you documented having fed her?

LETBY: It's not a coincidence?

JOHNSON: No, it's not an innocent coincidence?

LETBY: Yes, it is.

JOHNSON: Just going back to your defence statement, please, back to paragraph 92 that I asked you to read out before or I read out for you.

LETBY: Yes.

JOHNSON: "[Baby G] vomited at about 10.15. It wasn't as pronounced as the vomit on 7 September. I think her mother was there. The doctors were on the unit so I went to the nurses' station to ask the doctors who were doing the ward round to come and see [Baby G]. I did not shout for help because I did not think this was an emergency."

LETBY: Yes.

JOHNSON: Correct?

LETBY: Yes.

JOHNSON: You have sought to minimise what happened to [Baby G], haven't you?

LETBY: No.

JOHNSON: Let's just look at Dr Fielding's note, please, at tile 51:

"[Baby G] had an episode at about 10.20..."

So he puts a slightly different time on it to you.

LETBY: Okay.

JOHNSON: "... where she had two projectile vomits witnessed by nursing staff, after which she was apnoeic for about 6 to 10 seconds, went blue, sats down to 30."

LETBY: Yes.

JOHNSON: "Nurse called for help."

It's not the picture you paint, is it?

LETBY: No.

JOHNSON: And that is because you are seeking to minimise what you did?

LETBY: No. [Baby G] didn't need any intervention, it was a self-resolving apnoea.

JOHNSON: If we go to tile 111, please. This is you talking to your pal, [Nurse A], later that evening, isn't it --

LETBY: Yes.

JOHNSON: -- at 21.20?

LETBY: Yes.

JOHNSON: And you're talking about [Baby G] here, aren't you?

LETBY: Yes.

JOHNSON: But let's just go back, if we can, just to put this into context. Let's go back to 106, which I reminded you of earlier. So, "Due date today", you were telling [Nurse A]. Then [Nurse A] responded straightaway, 107:

"She likes to celebrate the big ones in style."

Followed immediately by:

"When was she screened if been cold for 24 hours?"

Yes?

LETBY: Yes.

JOHNSON: So that's a reference back to the low temperature, isn't it, that I say you deliberately misrecorded?

LETBY: No, it's due to having low temperatures over a 24-hour period that we saw on the charts that were in the yellow.

JOHNSON: Then 109:

"Due immunisations today too. I got her screened this morning after she vomited."

LETBY: Yes.

JOHNSON: And [Nurse A] responded:

"Was she still in 4 then?"

And you say:

"Yeah and had NN all weekend."

What does that mean?

LETBY: She had a nursery nurse looking after her all weekend.

JOHNSON: Yes. Then what did you write?

LETBY: "Looked rubbish when I took over this morning. Then she vomited at 9 and I got her screened."

JOHNSON: There are two lies there, aren't there?

LETBY: It's not a lie. I have miswritten the time, yes, it shouldn't be 9 am, but I don't believe she looked herself when I took over, no, and her mum commented on that.

JOHNSON: I'm sorry, but we established at the beginning of this process that [Baby G] was in good condition when you took her over.

LETBY: I don't remember being asked specifically about her colour.

JOHNSON: Well, let's look at Valerie Thomas' nursing note at tile 44. This is written just as you were coming on; is that right?

LETBY: Yes.

JOHNSON: "Observations satisfactory. Temperature: well wrapped up. Three feeds using own bottles. Completed bottles but had one NGT feed. To rest. Settled later after 21.15 feed. Fed by dad. Has passed urine and bowels open. Medication given as prescribed. Routine bloods taken this morning at 6 and sent to lab. For immunisations."

Let's look at the original just to check we've not missed anything. No suggestion of [Baby G] looking rubbish, to use your word, there, is there?

LETBY: No.

JOHNSON: Your nursing note at tile 48. Any suggestion that [Baby G] looked rubbish before she had this incident at 10.15 or --

LETBY: No, I've commented that she's pale. I have not used the word "rubbish" in clinical notes, no.

JOHNSON: Were you trying to persuade your mates on the unit that this was just one of those things that had happened to a child who was sickening for an infection?

LETBY: Can you say that again, please?

JOHNSON: Were you trying to create in the minds of your co-workers the impression that this was a child who was sickening for an infection?

LETBY: No, I wasn't suggesting anything; I was just saying what had happened.

JOHNSON: Did you put the time back to 9 o'clock because you were trying to create the impression that it happened nearer handover than it really did?

LETBY: No, it's an error, I agree it said 9 o'clock when it would have been 10, but that has nothing to do with handover or anything like that, no.

JOHNSON: With [Baby Q] in mind, if she really did look rubbish, why didn't you ask for a medical review before you fed her at 9.15?

LETBY: I don't think she did look rubbish as such. I put she's pale and I know mum had commented she didn't look as well as the previous day.

JOHNSON: But mum hadn't even arrived by the time you fed her, had she?

LETBY: No, not by the time I fed her, no.

JOHNSON: No, so you can't blame mum for this one.

LETBY: I'm not blaming mum at all. It was a comment she made.

JOHNSON: What comment did she make?

LETBY: [Baby G] didn't look as well as the previous day.

JOHNSON: I think that comment actually came from you, didn't it?

LETBY: No, I believe that came from mum.

JOHNSON: Let's go back, if we may, to Dr Fielding's note, then, at 10.30, tile 51.

We're going to have to go to the original because it's not in this.

At the bottom there, it's talking about [Baby G]'s distended abdomen. You see what looks like a shield has been --

LETBY: Yes.

JOHNSON: -- representing abdomen. So:

"Distended. Staff think perhaps more so than usual. Mum unsure."

LETBY: Yes.

JOHNSON: So what Dr Fielding has recorded is mum doesn't know; it's the staff, that's you, saying that [Baby G] looked worse.

LETBY: No, this is in regards to distension of the abdomen.

JOHNSON: Oh yes. It's all to do with the condition of [Baby G], isn't it?

LETBY: Yes.

JOHNSON: And Dr Fielding has written down at the time that this is a suggestion being made by you, not by mum.

LETBY: By staff, yes.

JOHNSON: Yes. But in your nursing note, this becomes a concern expressed by mum, not by you.

LETBY: Yes, and that's what I believe happened.

JOHNSON: Yes, and you are passing off responsibility to other people, aren't you --

LETBY: No.

JOHNSON: -- in the notes?

LETBY: No.

JOHNSON: Making it look like this information is all coming from other people and not from you?

LETBY: No.

JOHNSON: To make it look like you are doing nothing wrong?

LETBY: No.

JOHNSON: Whereas in fact, you are the person causing all these problems?

LETBY: No, I'm not.

MR JOHNSON: I'm going to go on to count 9, my Lord, so that may be a good point.

MR JUSTICE GOSS: It's a good point to break, certainly.

A 15-minute break then, please.

(In the absence of the jury)

MR JUSTICE GOSS: Just so there's no mystery, I've got a note from a juror about an upcoming medical appointment, which would mean, if it stands, we lose another day. I'm just seeing whether it can be worked round before I make a decision. Thank you.

(3.10 pm)

(A short break)

(3.25 pm)

(In the presence of the jury)

MR JOHNSON: If you'd just like to close up those interviews, please, and go back to the defence statement. We're at paragraph 93, please.

LETBY: Yes.

JOHNSON: From 93 to 96 inclusive, you deal with the incident that happened in nursery 4 at about 15.30 on the afternoon of 21 September.

LETBY: Yes.

JOHNSON: I'm just going to read parts of this out to you.

Starting at 93:

"Later, once [Baby G] became unwell, the doctors decided to put a cannula in and screen her. This was in nursery 4. I had another babies in nursery 4 so I did look round the screen to check on [Baby G] and saw that she was on her own, on the procedure trolley, with the monitor turned off. She looked blue and I called for help and started Neopuffing. [Nurse B] came to help. This was some hours after the vomit."

Is that how it appeared?

LETBY: Yes.

JOHNSON: And is it correct?

LETBY: Yes.

JOHNSON: Paragraph 94:

"I do not think that the probe that connected [Baby G] to the monitor was attached. Sometimes the doctors take the probe off or switch off the monitor when doing a procedure."

Is that what it says?

LETBY: Yes.

JOHNSON: And is this correct?

LETBY: Yes.

JOHNSON: 95:

"After this incident, [Nurse B] and I had a lengthy conversation about what had happened and I told her that the doctor had left [Baby G] on the trolley and this should be reported. [Nurse B] did not want to do this."

Is that what it says?

LETBY: Yes.

JOHNSON: And is it correct?

LETBY: Yes.

JOHNSON: Finally:

"Staff on the unit didn't like to report issues that the doctors got wrong."

Is that true of you?

LETBY: No, my general opinion is that staff in general did not comment on doctors doing anything wrong.

JOHNSON: What about you though?

LETBY: I don't know what I mean.

JOHNSON: Would you -- did it bother you, reporting doctors for getting things wrong?

LETBY: Yes.

JOHNSON: "Dr Gibbs had left [Baby G] on a procedural trolley with the monitor off and the screens up. [Nurse B] did not want to report this."

So is that what it says?

LETBY: Yes.

JOHNSON: Is it accurate?

LETBY: Yes.

JOHNSON: Right. When [Baby G] collapsed on this occasion, as a matter of fact, all other medical professionals, that is doctors and nurses other than you, were out of the room, weren't they?

LETBY: Yes, when I found [Baby G], yes, there was nobody there.

JOHNSON: That you would say is, what, an innocent coincidence?

LETBY: Yes.

JOHNSON: What you said to your counsel is that:

"I recall [Baby G] had been behind the screen for a prolonged period whilst the doctors were trying to cannulate her."

Is that right?

LETBY: Yes.

JOHNSON: And that you were preoccupied, I think you said, with the babies that you were caring for in the same nursery?

LETBY: Yes, I had no involvement with that cannulation procedure, no.

JOHNSON: No, but it goes a bit further than that, doesn't it, that you were preoccupied with the babies that you were caring for?

LETBY: Yes.

JOHNSON: Let's deal with that point first of all. If we go back to the neonatal review, please, and we're now in the second one for [Baby G], which is 8 and 9. It should hopefully be page 7 of 9. It has the correct time on it.

LETBY: Is this number 8?

JOHNSON: Yes, it should be 8 and 9, but it may just be in there as number 8. It's counts 8 and 9. The front it says "NNU review schedule, event 8".

LETBY: Yes.

JOHNSON: So if we go to page 7 of 9 if you would, please. We can see this desaturation is marked at line 287. Can you see that?

LETBY: Yes.

JOHNSON: We can see what you were doing in the preceding hour and a half on this page, can't we?

LETBY: Yes.

JOHNSON: So starting at 14.00 hours, 90 minutes before this event with [Baby G], you did observations for the child OR?

LETBY: Yes.

JOHNSON: And just to remind ourselves, OR, of course, is rooming with its parents?

LETBY: Yes.

JOHNSON: The next event is line 270 at 14.49. Again, OR?

LETBY: Yes.

JOHNSON: Rooming with parents, an e-prescription. An update made on the computer by you and Caroline Oakley?

LETBY: Yes.

JOHNSON: Caroline Oakley is, as a matter of fact, looking after two children in nursery 2. Then the event immediately or most proximate to [Baby G]'s collapse is at 272, isn't it?

LETBY: Yes.

JOHNSON: This is in relation to a child, RH.

LETBY: Yes.

JOHNSON: Who we can see from page 2 of 9 was one of Caroline Bennion's children in nursery 2.

LETBY: Right.

JOHNSON: Well, do you agree?

LETBY: Yes.

JOHNSON: So do we agree that you were not preoccupied with the children, your other children, in nursery 4?

LETBY: No, I don't agree with that. Just because we're not signing an observation chart doesn't mean that we're not doing things with the families or the baby.

JOHNSON: Okay. You say that it's common practice to look behind the screen; is that right?

LETBY: Yes.

JOHNSON: And I think the way you phrased it was:

"I happened to catch sight behind the screen and saw that [Baby G] was on her own."

LETBY: Yes.

JOHNSON: "She was dusky blue and not breathing."

LETBY: Yes.

JOHNSON: "The monitor was not on."

That's what you said.

LETBY: Yes.

(Pause)

JOHNSON: Can I just have a moment, please?

(Pause)

I'm afraid this screen has frozen.

(Pause)

Sorry, usually this wouldn't bother me, my Lord, but there are so many references here that I can't do it without my note. I'll try a different machine.

(Pause)

What you said in answer to questions from your own counsel was that when you saw [Baby G], she was dusky blue and not breathing; is that true?

LETBY: Yes.

JOHNSON: You said the monitor was not on.

LETBY: No, it wasn't on.

JOHNSON: That you picked her up?

LETBY: Yes.

JOHNSON: You put her in a cot?

LETBY: Yes.

JOHNSON: And you started to Neopuff her?

LETBY: Yes.

JOHNSON: Then you called for help?

LETBY: Yes.

JOHNSON: You said that the Neopuff wouldn't have stretched to the procedure trolley?

LETBY: No, that's right.

JOHNSON: You said that [Nurse B] froze and that she went to get Caroline Bennion?

LETBY: Yes.

JOHNSON: Was [Nurse B] the first person that answered your call for help?

LETBY: Yes.

JOHNSON: And you said that you were very concerned by what had happened?

LETBY: Yes.

JOHNSON: That the child shouldn't have been left behind the screen?

LETBY: No.

JOHNSON: Shouldn't have been left on a procedure trolley?

LETBY: No.

JOHNSON: And neither should she have been left without a monitor on?

LETBY: That's right.

JOHNSON: And that you wanted to put a Datix form in?

LETBY: Yes.

JOHNSON: But [Nurse B] said she would deal with it?

LETBY: Yes.

JOHNSON: One of the things you don't mention in your defence statement that I have just taken you through is moving [Baby G] from the procedure trolley to the cot.

LETBY: Okay.

JOHNSON: Why is that?

LETBY: I -- I can't answer that now. I just know at the time that [Baby G] was on the procedure trolley and it wasn't near the Neopuff on the wall, I had to move her.

JOHNSON: It wasn't that you had moved her to the cot and you had taken the opportunity to sabotage her --

LETBY: No.

JOHNSON: -- knowing that the doctors had left her behind the screen?

LETBY: No, [Baby G] was on the trolley and I put her into the cot.

JOHNSON: You took advantage of a situation that presented itself, didn't you?

LETBY: No.

JOHNSON: Just let's think about that. You've already told the jury that you were in nursery 4.

LETBY: I can't recall specifically what I was doing. She was there for a prolonged period of time behind the screen, but yes, I was in and out of the nursery, yes.

JOHNSON: I'm sorry, but what you said in the defence statement was this, and we've been through it once but I'll go through this little bit once more:

"Later, once [Baby G] became unwell, the doctors decided to put a cannula in and screen her. This was in nursery 4."

LETBY: Yes.

JOHNSON: "I had other babies in nursery 4, so I did look round the screen to check on [Baby G]."

LETBY: Yes.

JOHNSON: You said earlier, when I was asking you questions, that you were engaged with those other children in nursery 4 --

LETBY: Yes.

JOHNSON: -- and that was the reason why we went through the other material from the neonatal review to see what, if any, other children elsewhere you might have been dealing with in the lead-up to this time at 15.30.

LETBY: Yes.

JOHNSON: We have established, haven't we, that you had no other responsibilities elsewhere but in nursery 4?

LETBY: Yes.

JOHNSON: And so when Drs Gibbs and Harkness were cannulating [Baby G], you were in the room?

LETBY: At times, yes.

JOHNSON: No, all the time.

LETBY: No.

JOHNSON: Why were you out of the room?

LETBY: Well, I've done things here for another baby at 15.00 -- I wouldn't be predominantly in the nursery the whole time.

JOHNSON: What have you done for another baby at 15.00, an entry on the fluid balance chart at line 272?

LETBY: Yes.

JOHNSON: That's it. How long does that take, 5 minutes?

LETBY: I can't say without looking at what it was that I did.

JOHNSON: Very well. Could we have J29112 on the screen, please, Mr Murphy.

You tell us.

(Pause)

I will describe to you what it is and we'll try and get it on the screen. It is a neonatal fluid balance chart, the like of which we have seen in many other cases. So I'll give you an example. If we were to go behind divider 5 in the case of [Baby E], go to page 2719, the only part of this -- what I'll do is I'll show it to Mr Myers as I'm describing it, so if there's any suggestion that I am misdescribing anything...

(Pause)

If we're looking at the page that's in the [Baby E] file, 2719, the only part of the fluid balance chart that's filled in for the child RH are the readings in the top fluid section; okay?

LETBY: Okay.

JOHNSON: So in [Baby E], it says "Babiven".

LETBY: Yes.

JOHNSON: And the only readings -- so if we take -- I'll describe it by reference to this one. Go to 15.00, which is what we're talking about for RH.

LETBY: Yes.

JOHNSON: It's the 7.6, 88.1 and then the 199 over what looks like a 1.

LETBY: Okay.

JOHNSON: And that is the only thing --

LETBY: Okay.

JOHNSON: -- that is filled in on that hour for that child at that time.

LETBY: Okay, thank you.

JOHNSON: How long does that take? It's taking a reading off a pump, isn't it?

LETBY: Yes. A matter of minutes.

JOHNSON: A matter of a couple of minutes?

LETBY: Yes.

JOHNSON: We've found it now. Can we scroll so that we can't read the name? Perfect, thank you. Put it on the screen.

It's my fault for not teeing this up, I'm sorry.

So can you see 15.00 hours under where it says 16.5ml per hour?

LETBY: Yes.

JOHNSON: That's what I have just described to you, and if Mr Murphy would scroll so we can see further down the page to see your initials -- there.

LETBY: Yes.

JOHNSON: Okay? So do you accept what I described --

LETBY: I do, yes.

JOHNSON: -- as a fair description of what we see?

LETBY: Yes, thank you.

JOHNSON: And do you accept that that is the work of 5 minutes at most?

LETBY: Yes.

JOHNSON: At 3 pm?

LETBY: Yes.

JOHNSON: So it follows that if the time is correct you were in nursery 4 for at least 25 minutes prior to noticing that [Baby G] had collapsed?

LETBY: I can't say that definitively, no.

JOHNSON: Well, what time frame would you put on it?

LETBY: I can't recall specifically. I was in and out of the nursery throughout the day.

JOHNSON: What else were you doing?

LETBY: There are many things that we have to do on the unit that is not based purely at the baby's cot side.

JOHNSON: But I'm going to suggest to you, you see -- do you remember Dr Gibbs and Dr Harkness said that if the nurse with responsibility for the child -- and by this stage it was [Nurse B], wasn't it?

LETBY: Yes.

JOHNSON: If she wasn't in the room, they would have told another nurse in the room that they had finished?

LETBY: Yes, that's what they said, yes.

JOHNSON: And if they're right about that, that would have been you, wouldn't it?

LETBY: Yes, but that's not right.

JOHNSON: How do you know that's not right?

LETBY: I do not recall at any time being told by the doctors that they had finished with [Baby G] and that I needed to put her back in the cot.

JOHNSON: That's why I'm suggesting to you that you took advantage of a situation when other people weren't around.

LETBY: No, that was how I found [Baby G].

JOHNSON: And because you hadn't worked out exactly what you were going to say about this, just going back to your defence statement, that's why you don't mention at all putting [Baby G] back into the cot, do you?

LETBY: I think that's a minor issue in terms of what was actually going on with [Baby G] at that point. The fact was she had been left, inappropriately, on a trolley.

JOHNSON: Well, that would be a terrible thing to do if no one was in the room and no one had been told, wouldn't it?

LETBY: Yes.

JOHNSON: Whereas if you were in the room and you had been told, it would put it into rather a different --

LETBY: No, I disagree. The screens were still around [Baby G].

It would be up to the doctors to put those screens away and ensure that [Baby G] was back in a safe position in the cot with her monitor on.

JOHNSON: That's why I'm suggesting you took advantage of a situation that presented itself and you sabotaged [Baby G]?

LETBY: I disagree.

JOHNSON: For a third time?

LETBY: No.

JOHNSON: Well, let's move on to the case of [Baby H], please. If we take that picture down, Mr Murphy, thank you.

Do you remember [Baby H]?

LETBY: Yes.

JOHNSON: Very memorable, wasn't she?

LETBY: Yes.

JOHNSON: Because of the chest drains?

LETBY: Yes.

JOHNSON: You know, don't you, that we make two allegations of attempted murder against you --

LETBY: Yes.

JOHNSON: -- counts 10 and 11, which relate to two episodes of cardiac arrest in the early hours of the 26th and 27 September?

LETBY: Yes.

JOHNSON: Do you have a memory of [Baby H] independently of the notes?

LETBY: Yes, some.

JOHNSON: You remembered one particular detail, didn't you, that someone had had to courier chest drains from the Liverpool -- from Arrowe Park, I think it was, you said?

LETBY: Yes, after the event, not on the night of [Baby H]'s collapse.

JOHNSON: Is that a reflection of how good your memory is that you can remember little details like that?

LETBY: Yes, it was discussed widely on the unit that it was not acceptable and we needed to get some couriered in, yes.

JOHNSON: Did you Datix that?

LETBY: I can't recall. I'd have to check.

JOHNSON: You had one of the handover sheets for [Baby H], didn't you, under your bed?

LETBY: Yes.

JOHNSON: And you told the jury on 15 May that you didn't know it was under your bed?

LETBY: No.

JOHNSON: That's not true, is it?

LETBY: Yes, it is.

JOHNSON: Another thing that you said in answer to questions from your counsel -- and if we could just put this up, please: it's in the [Baby H] sequence and it's tile 309.

Do you remember this one to Yvonne Griffiths:

"Really nice to hear as I gather you are aware of some of the not-so-positive comments that have been made recently regarding my role which I have found quite upsetting"?

Do you remember that?

LETBY: Yes.

JOHNSON: Do you remember what it was you told the jury that lay behind that?

LETBY: It was in relation to myself and Shelley Tomlins, who were newly QIS trained, having babies like [Baby H].

JOHNSON: Yes. I think the way you put it on 15 May was that:

"Some of the existing staff were not overly happy with people like myself and Shelley Tomlins having babies in nursery 1."

LETBY: Yes.

JOHNSON: Who?

LETBY: I can't recall specifics, but it was a general conversation with certain band 6s at that time.

JOHNSON: Which band 6s?

LETBY: I can't recall specifically who.

JOHNSON: Are you not naming names in case they would contradict you?

LETBY: No.

JOHNSON: I think you also said that this bad feeling, if that's the right way of putting it -- that's my word, not yours, okay, I'll use that phrase -- this bad feeling stopped after this point; is that right?

LETBY: I recall this was specifically in regards to [Baby H], yes.

JOHNSON: So again, it's my word, so I'll take responsibility for it, but any bad feeling from the band 6s was simply related to [Baby H], was it?

LETBY: No, there was some general concern, but this that I'm talking about here is specific to [Baby H], yes.

JOHNSON: Right. Let's just look at who the band 6s were who were on if we can. If we go to tile 177, please. This is the first time [Baby H] collapsed, this shift.

LETBY: Yes.

JOHNSON: It's not [Nurse A], is it, she's your mate?

LETBY: No, I'm not talking about the staff that were on duty that night.

JOHNSON: Well, how would anyone who wasn't on duty know that you had [Baby H]?

LETBY: It was commented on by the day staff.

JOHNSON: How would the day staff -- why would the day staff be interested that you had [Baby H]?

LETBY: The day staff allocate who's looking after which patients.

JOHNSON: So because they don't want you to have [Baby H], they allocate you [Baby H]?

LETBY: It wasn't a don't want, it was a comment that potentially other people needed the experience more than Shelley and I did.

JOHNSON: Right. Let's look at this. Who of the nurses on needed the experience more than you or Shelley?

LETBY: I'm not saying that it happened on this shift here.

JOHNSON: But you are: you said it was all to do with [Baby H].

LETBY: Yes, but not this specific shift and time.

JOHNSON: Are you now trying to avoid answering the question when I ask you for particulars of an answer that you gave earlier in answer to your counsel?

LETBY: No.

JOHNSON: Because you're lying about that, aren't you?

LETBY: No.

JOHNSON: Were people talking about you being associated to the collapses of lots of children at different times?

LETBY: No, not at all.

JOHNSON: Is that the negative comments that you're talking about at tile 309?

LETBY: No.

JOHNSON: Were people starting to notice that you were associated to lots of different collapses?

LETBY: No.

JOHNSON: Can we go back to your defence statement, please.

Paragraph 97:

"I did nothing to harm [Baby H]. I have little recollection of matters relating to [Baby H] outside what is recorded on the medical notes. I do recall that she was not sedated and she moved a lot."

Is that right?

LETBY: Yes.

JOHNSON: Is that an explanation for why her drains might move?

LETBY: That's not what I'm saying. I'm saying that I recall that she did used to move around, yes.

JOHNSON: Yes, but -- we can all see that that's what you say.

What I'm interested in, and what I would be very much obliged if you would answer, is the reason why you put that in this document.

LETBY: That potentially, yes, she could have dislodged one of the drains.

JOHNSON: Why not just agree when I ask you the question the first time?

LETBY: Okay.

JOHNSON: "I had never seen a chest drain on a baby at the Countess of Chester before [Baby H] and I've never seen three chest drains on a baby at the same time. I'm not sure how familiar all the staff were at the Countess of Chester with chest drains. Fluid came from the chest drains for the whole of her time at the Countess of Chester NNU. The drains were only taped in place, so they were not as secure as if they had been sutured."

That's the same point, isn't it?

LETBY: Yes.

JOHNSON: "I question how familiar the doctors were with the use of chest drains."

What grounds did you have for questioning how familiar the doctors were with chest drains?

LETBY: There was often discussion amongst the doctors about how to manage drains and they often sought advice from the consultants about the drains.

JOHNSON: Right, so by doctors you're referring to non-consultants?

LETBY: Yes, and they would ask advice from their senior colleagues, yes.

JOHNSON: Registrars and house officers regularly ask advice from their senior -- for guidance from their senior colleagues, don't they?

LETBY: Um... not in how to conduct a procedure, no.

JOHNSON: "Dr Harkness had to receive advice over the phone from Dr Gibbs when fitting the chest drain."

LETBY: Yes.

JOHNSON: So that's the first of the chest drains, isn't it, the first of the three?

LETBY: I'm not sure specifically which drain.

JOHNSON: We know Dr Jayaram -- we'll come to it, but I think Dr Jayaram inserted the second chest drain; do you remember?

LETBY: I can't remember who inserted which drain now off the top of my head, no.

JOHNSON: Paragraph 100:

"[Baby H] had a number of collapses over the days 24-27 September, although I can't remember the detail of these."

You say that you don't remember any specific episode of desaturation during the night shift of the 25th to the 26th. But then when we come to what is our count 10, you say at your paragraph 102:

"I do recall that on..."

Sorry, I'm getting slightly confused here. 101 is -- if we just go to jury bundle 1 and make sure we don't make a mistake and go to the indictment. It's page 4, please.

LETBY: Sorry, do I have that?

JOHNSON: You do. It's in jury bundle 1, the one with the big blue 1 on it. Page 4.

LETBY: Under divider 4?

JOHNSON: No, it's divider 1, page 4, please. Do you see count 10 is an allegation of attempted murder on 26 September?

LETBY: Yes.

JOHNSON: So you just keep that open but go back to your defence statement, please. Paragraph 101:

"I don't recall any specific episode of desaturation during the night shift of the 25th to 26 September."

Is that right?

MR JUSTICE GOSS: Sorry, there's just a distraction there.

Someone inadvertently came in the back door but they've gone, or they opened the door. Right. They intentionally opened the door, but inadvertently that door.

MR JOHNSON: Yes. Okay, let's just rewind slightly.

Count 10, okay, have you got that?

LETBY: Yes.

JOHNSON: This is an allegation of attempted murder in the early hours of 26 September.

LETBY: Yes.

JOHNSON: Do you follow?

LETBY: Yes.

JOHNSON: Your paragraph 101 says:

"I don't recall any specific episode of desaturation during the night shift of the 25th to 26 September."

LETBY: Yes.

JOHNSON: Right. Then count 11 is a very similar allegation of attempted murder, albeit it's on the next night shift; so this is 27 September.

LETBY: Yes.

JOHNSON: Okay. These are two night shifts, consecutive night shifts that you were working?

LETBY: Yes.

JOHNSON: And so, so far as count 11 is concerned, what you say is this:

"I do recall that on the 26th to 27 September [Baby H] deteriorated rapidly. I was with [Nurse A]."

Do you agree?

LETBY: Yes.

JOHNSON: Then you say as further detail:

"In my memory, both nights merge into one. I cannot be specific about what I did during either of the collapses. However, on the 26th to 27 September [and this is count 11] I was looking after another severely disabled baby and my focus would have been on him. I may have signed for some medication for [Baby H], but I did not administer this."

Is that correct?

LETBY: Yes.

JOHNSON: So is that what the document says?

LETBY: Yes.

JOHNSON: And is it a correct reflection of what you say about this?

LETBY: No, because I now know looking at paperwork that the baby was born later into the shift.

JOHNSON: Right. So the severely disabled baby that you are talking about was born later?

LETBY: Yes.

JOHNSON: Okay. Off the top of your head, do you know the baby's initials?

LETBY: Yes.

JOHNSON: Would you tell them to us?

LETBY: LO.

JOHNSON: LO. Then at paragraph 104:

"I do not recall that any discolouration to [Baby H] was the same as that on other babies."

LETBY: Yes.

JOHNSON: Then 105:

"I do not think [Baby H] was given surfactant at birth, although she should have been."

LETBY: Yes.

JOHNSON: Is that right?

LETBY: Yes.

JOHNSON: Now, other than the reference to LO and to your mistake, is there anything else in that material that I have just read to you with which you now disagree?

LETBY: No.

JOHNSON: So let's examine the occasions one by one, if we may, please, then. So far as count 10 is concerned, [Baby H]'s desaturation and cardiac arrest in the early hours of 26 September 2015, are you suggesting that staffing levels caused or contributed to that event?

LETBY: No.

JOHNSON: Are you suggesting that medical competence or incompetence contributed to it?

LETBY: I'm not sure of the exact details of this date.

JOHNSON: All right. What about anybody's mistakes?

LETBY: I question whether some of the drains were securely put in.

JOHNSON: Okay. So an issue about the security of the drains?

LETBY: Yes.

JOHNSON: You didn't deliberately misplace the drains, I don't suppose, did you?

LETBY: No.

JOHNSON: If they became displaced, you say that's down to the default of somebody who fixed them?

LETBY: No, potentially it is, yes. I've only ever seen them stitched in, I've never seen them just stuck down with tape.

JOHNSON: Do you agree that [Baby H] was born in good condition?

LETBY: I can't comment on that right now without seeing...

JOHNSON: All right. We'll help you. Tile 5, please. It says there, this is the note of Dr Matthew Neame, don't let the avatar deter you. It says:

"Born in good condition."

LETBY: Yes.

JOHNSON: Do you dispute that?

LETBY: No.

JOHNSON: After all these events, or the two events in particular with which you are charged, do you dispute that [Baby H], her condition, improved dramatically once she got out of the Countess of Chester?

LETBY: No, her condition did improve.

JOHNSON: Do you accept the evidence of Professor Arthurs, the radiologist from Great Ormond Street, that there is nothing concerning about the position of the chest drains?

MR MYERS: That's a matter of expert evidence, my Lord, and that's the sort of question raised before. We have dealt with that and that is for the jury in due course.

It's not for Ms Letby to talk about radiology like that, in my submission.

MR JOHNSON: There are comments in the defence statement about things like this, so --

MR JUSTICE GOSS: I think you could ask Ms Letby whether, from her own direct knowledge of chest drains, and what she could see in relation to these chest drains, whether there was anything untoward. She's referred to them not being stitched in, she's always seen them stitched in before. Anything else? That's the question.

LETBY: No, nursing staff can't interpret X-rays or anything like that, so I can't comment on that.

MR JUSTICE GOSS: That's for radiologists?

LETBY: Yes.

MR JOHNSON: Right. I'll just remind you of some of the chronology, like Mr Myers did. It's a slight repetition, but it may help you.

The first chest drain was inserted on 24 September by Dr Harkness at about 10.0 hours. Okay?

LETBY: Okay.

JOHNSON: If anyone's looking for it, it actually isn't in the sequence of events and that's why I mention it. That was to deal with a tension pneumothorax. Right?

LETBY: Okay.

JOHNSON: The night shift of the 25th into 26 September -- sorry, the 24th into the 25th, I beg your pardon, you were [Baby H]'s designated nurse, weren't you?

LETBY: I take your word for that. I can't say from memory now.

JOHNSON: All right.

LETBY: Sorry, the dates...

JOHNSON: It's all right, don't worry. It is confusing. No one would dispute that. But I'll just help you with tile 40 of the sequence.

Here we see that you were the designated nurse on this shift. We see your retrospective nursing note written at 08.21 on the morning of Friday the 2nd.

LETBY: Yes, thank you.

JOHNSON: That's why I'm suggesting that you were [Baby H]'s designated nurse.

LETBY: Yes, I accept that, thank you.

JOHNSON: Just to remind you, it was on this shift at 01.45 that your friend, Dr Ventress, inserted a butterfly needle.

Do you remember that?

LETBY: Yes.

JOHNSON: Dr Verghese intubated [Baby H] at 02.45?

LETBY: Okay.

JOHNSON: And Dr Jayaram, at about 03.00, inserted the second chest drain in the seventh intercostal space.

LETBY: Okay.

JOHNSON: Do you remember that? And there was quite a lot of evidence about whether or not that was an appropriate place to put a chest drain.

LETBY: Yes.

JOHNSON: That it was not a pigtail chest drain, it was a more old-fashioned version of a chest drain.

LETBY: That's right, yes.

JOHNSON: Okay. So that just puts it into context. I'm going to move on now to ask you about what happened on the following night shift of the 25th into the 26th. I can start from the beginning tomorrow, my Lord, or deal with a few preliminary questions now in the remaining 2 minutes.

MR JUSTICE GOSS: No, I think we'll do it of one piece tomorrow for the sake of a few minutes.

MR JOHNSON: Yes.

MR JUSTICE GOSS: So we'll come back to that tomorrow morning. 10.30 tomorrow then, please, members of the jury. I remind you of your obligations not to conduct any research about anyone or anything to do with this case or to communicate with anyone in any way about anyone or anything to do with this case. Thank you very much. 10.30 tomorrow. Of course, you know from the sheet of paper, tomorrow is our last day of this week, we're not sitting on Friday, so tomorrow is the last working day here for you. Thank you.

(In the absence of the jury)

Housekeeping

MR JUSTICE GOSS: In relation to a potential non-sitting day at the end of next week, I'm seeing if that can be re-arranged, so I can't give an answer in relation to that. But could I just ask you, Mr Johnson, about how much longer you anticipate your cross-examination is going to be, roughly?

MR JOHNSON: Well, at the moment I've had just under 3 days.

MR JUSTICE GOSS: It doesn't matter how long.

MR JOHNSON: I'm just trying to work it out, sorry. I think I'm pretty much about halfway.

MR JUSTICE GOSS: So 2 days, about, of next week?

MR JOHNSON: Yes, about that.

MR JUSTICE GOSS: All right. Well, the potential non-sitting day, or at least part of it, if not all of it, would be Friday of next week as things presently stand. So that's where we are.

MR JOHNSON: Yes. Given what Mr Myers raised at a previous hearing, that may not actually be a wasted day from the court's point of view.

MR JUSTICE GOSS: Exactly, because Monday is a public holiday.

MR MYERS: What Mr Johnson is referring to is when Ms Letby has concluded her evidence, we will need some time with her before we can proceed from there, so it may well be that if we're not sitting that day but we're at that point, that would be very helpful if that's how it works out.

MR JUSTICE GOSS: If that's how it works out, yes.

Thank you very much, both of you. All right.

(4.16 pm)

(The court adjourned until 10.30 am on Thursday, 25 May 2023)